



*Empowered lives.
Resilient nations.*

After Action Review

UNDP Response to the Ebola Crisis

Conducted and written by an independent consultant



Photos: UNDP/Lesley Wright

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EXECUTIVE SUMMARY

This After Action Review covers UNDP's response to the Ebola crisis from its onset in early 2014 until the International Donor Conference in July 2015. An After Action Review is not an evaluation, but a 'lessons learned' process that relies primarily on capturing the first-hand accounts of people involved in an exceptional event.

The Ebola crisis was the first high-impact crisis involving UNDP since the 2010 Haiti earthquake, and although it resulted in fewer deaths, it required in many ways an even more complex response. As a large-scale health emergency it did not easily fit into the established coordination and response systems of the international community: it was too large and urgent for a development response, and yet it was also too sharply focussed and technical for a conventional humanitarian response. Furthermore, as a multi-country crisis, it could not be addressed through the usual mechanisms that are structured on a country basis.

To overcome these systemic obstacles and to immediately mobilize resources from assessed contributions, the UN Secretary-General created a unique United Nations body, UNMEER, to coordinate and manage the Ebola response after the unanimous adoption of General Assembly resolution 69/1, and the adoption of Security Council resolution 2177 (2014). UNDP was assigned a support role (not coordinating), until December 2014 when UNDP was requested to lead the UN system's initiatives on Ebola-related recovery. UN Resident Coordinators resumed overall coordination at country levels from July 2015 onwards when UNMEER closed.

UNDP's role evolved as the international community continued to learn about how best to respond to Ebola. The response phase was characterised first by medical (isolation and treatment) responses, and later by strengthening of capacities within and at the boundaries of the greater health system including logistics, health worker training and hazard pay, public awareness, border management, and government coordination. In this early period of rapid change and in the shadow of UNMEER, UNDP had difficulty developing a clear overarching statement of its role, and was further disadvantaged by the unfortunate timing of a major organisational restructuring that was being implemented right at the peak of the crisis – as a consequence of which, roles and functions within UNDP HQs and at regional level were being reconfigured.

From December onwards attention turned to recovery, to analysing the longer-term socio-economic impacts of the epidemic, and finally to planning and programming for getting back onto the pathway to sustainable development after Ebola. This was the moment when UNDP's development mandate came back to the forefront, the role of UNDP became easier to articulate, and UNDP's normal systems were fully engaged. Despite UNDP's substantial efforts to help host governments plan, programme and convene the donors around early recovery, recovery was not attractive to donors and post-Ebola recovery financing has been disappointing.

WHAT WENT WELL?

Three management decisions stand out as having been instrumental in UNDP's response: (a) the activation of the L3 Crisis Board and the early appointment of a Senior Coordinator helped mobilise everyone around UNDP's response, (b) the decision to assume the responsibilities and risks of Ebola worker payments became the central programmatic contribution to the early response phase, and (c) the fielding of a senior mission to the field was the turning point in providing HQs and the field with a sense of common purpose.

In terms of people management: (a) the deployment of staff to support the field was more effective than the deployment of consultants, (b) duty of care was a high priority and well-managed through the upgrading of UN clinics and country-level special measures, and (c) mid-level officers were given the trust and the management space to take risks and perform well.

UNDP made three landmark contributions that shaped the global response: the Ebola Worker Payment Project, the Ebola Recovery Assessment, and the International Donor Conference. These were complemented by less visible but equally effective analytical and programming work at country level, and all were well supported by strong operational coordination.

WHAT COULD HAVE BEEN IMPROVED?

For several reasons, it was extraordinarily difficult, particularly in the early days, to forge a single clear statement of UNDP's role and strategy – a narrative that the institution could rally behind and then organise around. The whole scope of Ebola was just too difficult to capture, as it appeared different in the medical/response stage from the early recovery stage, it was different at national level from the institutional level, and each country had its unique characteristics that defied a single regional argument. In the absence of this guiding star, the first months of communications, resource-mobilisation and programming each had difficulty getting coherence and focus.

Partly as a consequence of this, sometimes UNDP staff working on related tasks – either in different divisions of HQs or in the field – did not manage to fully connect up to their colleagues working on the same or similar issues. On occasions where the 'last few dots were not connected' there were examples of work that did not achieve its full potential result. Among these we can identify strong field analysis that did not always connect with HQs planning, project proposals that did not connect with donor interests, and the initial advocacy for early recovery that did not sway UNMEER. What seems to have been missing was both a narrative and a mechanism to 'connect the last few dots', and in cases where 'dots were connected' (Ebola worker payments, the International Donor Conference, the UN clinics) both narrative and mechanism were in place.

Both at the Crisis Board, and in other places throughout the response, early management behaviour was decisive, quick and confident. But as time went on this early sharpness and focus was diluted, as fewer decisions were needed the decisiveness gave way to information sharing, and coordination became slower as the desire to consult broadly and to develop consensus prevailed.

Finally, while the SURGE plan was strong and the initial staff deployments were effective, the second wave of mainly consultant employees was delayed. Since the first people deployed have a significant strategic advantage, this delay of 4-6 weeks at a pivotal moment of the response cost UNDP in the field in terms of reduced relevance, leverage and access. Once deployed, the SURGE teams could have been better integrated into the country offices, and the wrap-up of SURGE deployments could also have been more deliberately managed in conjunction with the country offices.

HIGH-LEVEL RECOMMENDATIONS FOR A SIMILAR LARGE-SCALE EMERGENCY

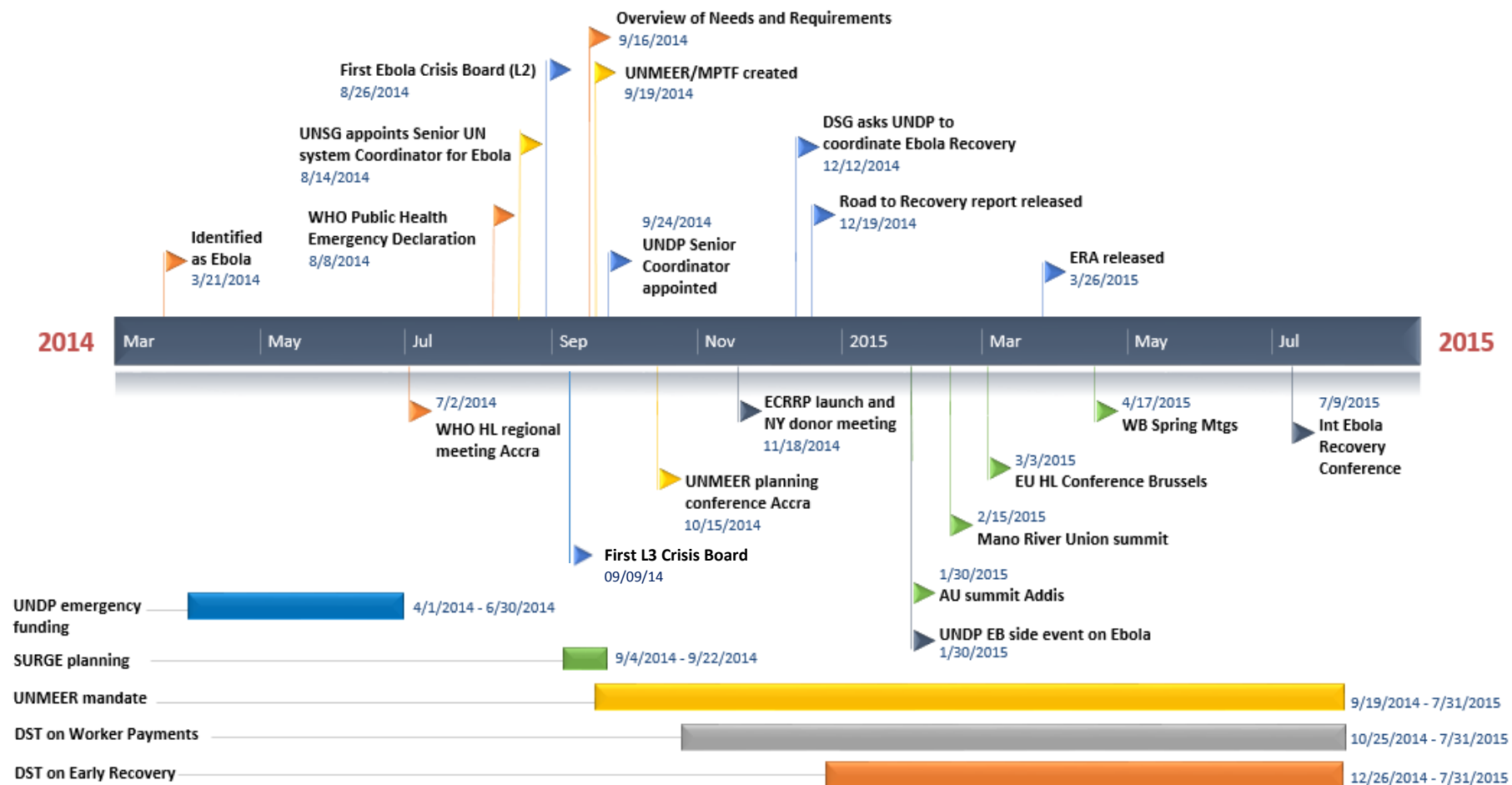
From the many recommendations made by staff and reflected in the report, four in particular stand out:

1. **Being quick is important:** mobilising the organisation, appointing a senior coordinator, determining UNDP's role and a key narrative, freeing up programme resources, empowering and fielding staff – these all need to be done quickly and firmly if UNDP is to secure and hold its place as coordinator and champion in an Ebola-like situation. In order to be quick, UNDP also needs to be ready, and for this both contingency planning and preparedness are key enablers;
2. Even when the pressures come from the Security Council and the Secretary-General, it is important to **remain country-led, response to host governments and to use existing systems**. Working around these can be expedient in the short-term but is less effective and sustainable in the long-term;
3. Similarly, **avoid skipping steps**: work on analysis, vision, strategy, programming and projects needs to be linked so that UNDP's work has context and quality. Ideally these steps should be sequenced, but in an emergency such work can take place in non-linear ways through linked teams working on these components in parallel.
4. Finally, **stick to what UNDP does best** (defined broadly and leaving scope for innovation); and if there is no clear role for UNDP that does not encroach on the mandates of others, then consider limiting engagement to overall coordination and the provision of quality common services to enhance the work of others.

ACRONYMS

ASG	Assistant Secretary-General
BERA	Bureau for External Relations and Advocacy
BMS	Bureau for Management Services
BPPS	Bureau for Policy and Programme Support
CD	Country Director
CO	Country Office
CRU	Crisis Response Unit
DST	Development Solutions Team
ERA	Ebola Recovery Assessment
HCT	Humanitarian Country Team
MPTF	Multi-Partner Trust Fund
PPERW	Payments Programme for Ebola Response Workers
RBA	Regional Bureau for Africa
RC	UN Resident Coordinator
RO	Regional Office
SG	Secretary-General
SOPs	Standard Operating Procedures
SRSB	Special Representative of the Secretary-General
UNCT	UN Country Team
UNMEER	United Nations Mission for Ebola Emergency Response

TIMELINE OF THE EBOLA RESPONSE



CONTEXT

1. NEITHER A HUMANITARIAN NOR A DEVELOPMENT CRISIS

UNDP's Ebola response, including the way UNDP approached coordination, was complicated by three contextual factors. First of all, the scale and character of this health emergency were without precedent – it did not fit easily into the established interagency systems of either development or humanitarian coordination,¹ and the natural lead technical agency WHO was not operational at the scale required. This coordination conundrum was addressed in September 2014 when a unique mechanism UNMEER² was established under the authority of the UN Secretary-General (SG). UNMEER had a mandate to address both the emergency and the ongoing delivery of essential services, and Resident Coordinators (RCs) were explicitly instructed by the SG to report to the Assistant Secretary-General (ASG)-level Ebola Crisis Managers on all matters related to Ebola, but UNMEER did not fully engage the essential services part of its mandate. As a result, **the job of ensuring the linkages between the immediate response and development domains was left in a 'mandate grey zone' until the end of December**, when responsibility for leading UN system initiatives for recovery was explicitly delegated by the Secretary-General to UNDP.

"UNMEER leaders emphasised that emergency response is all about effectiveness and not about efficiency." Review respondent.

2. A MULTI-COUNTRY CRISIS WITH ASYMMETRICAL SITUATIONS AND CAPACITIES

The second complicating factor was that, as an emergency with global ramifications, regional (transboundary) impacts, distinct national-level responses and sub-national activities, coordination needed to take place at multiple levels. Given the geographic complexity, within UNDP there was a particular **challenge with developing a clear strategy for UNDP's engagement which encompassed both the overall response and the very different needs and approaches of the three most affected countries**.³ Regional coordination was additionally complicated because UNMEER based themselves regionally in Accra, WHO's regional office was in Guinea, and UNDP regional office for Africa based in NY with a regional hub in Addis and a regional platform in Dakar.

3. UNDP RESTRUCTURING IN THE MIDDLE OF THE CRISIS

Finally, the moment when this building health crisis boiled over into a full-blown global emergency was precisely the moment⁴ when UNDP was implementing a major internal restructuring, including a widespread reassignment of HQ staff and functions, the redefinition of bureaux mandates including the separation of the Crisis Response Unit (CRU) from the former Bureau for Crisis Prevention and Recovery. For some time into the crisis response, there was **genuine confusion over who does what, who reports to whom**, and some staff were concerned about retaining their jobs in the face of significant cutbacks. This disruption seems to have affected some work units

"This was a regional problem with national responses." Review respondent.

¹ It was observed by several senior staff that the way Ebola landed somewhere in-between the development and humanitarian domains continued to colour decision-making throughout the whole response, as some key actors believed that the more humanitarian principles of effectiveness and saving lives should prevail, while other actors just as reasonably advocated for efficiency and sustainability, and strengthening of long-term systems. Neither emphasis was clearly right or wrong, one perspective or the other might be more relevant depending upon the date, the location and the issues -- but the two approaches are fundamentally different and this itself could be a source of misunderstanding and confusion within UNDP as well as between UNDP and other agencies.

² This AAR does not examine whether UNMEER was a necessary mechanism or whether it was set up appropriately, although the AAR will consider the UNMEER-UNDP relationship in different ways.

³ This problem of working on three related national situations and transboundary issues is problematic for all development and humanitarian agencies that are organised on a country basis.

⁴ UNMEER was created on 19 September, and UNDP's new structure became operational on 1 October 2014.

more than others – particularly the newly-consolidated BPPS underwent the most dramatic changes, while BOM/BMS, BERA and the Regional Bureaux reorganised but retained the same basic functions.⁵ It is to be noted that some managers found that this moment of fluidity permitted new relationships to be created, old bottlenecks to be circumvented, and helped them be more effective.

4. RISK MANAGEMENT WAS A PROMINENT FEATURE

Throughout the course of this After Action Review, interlocutors in HQ and in the field have drawn attention to two other themes threaded throughout UNDP's response. Firstly, **risk management was very explicit and prominent at every stage and in every location:** (a)

"You cannot fight a war with peacetime rules." Review respondent.

managing the health and safety risks of staff and consultants deployed into the epicentre of an epidemic became a huge preoccupation of the Regional Bureau and of BMS and correctly drove many management decisions; (b) managing the fiduciary risks of incentive payments and eventually mobile payments on a scale never seen before in West Africa, and the reputational risks of guaranteeing delivery; (c) managing the reputational risks to UNDP of either not being sufficiently present or sufficiently relevant particularly after UNDP's normal coordination functions were effectively sidelined by the creation of UNMEER; and (d) recognising that in an epidemic emergency with global implications there is a hierarchy of risks, and that some risks are far greater than the risks of non-compliance.⁶

5. THE MULTIPLE ROLES OF A RESIDENTIAL COORDINATOR

The second theme that emerged from interviews was the 'multi-hatting' challenge that UNDP and in particular the UNRC faces as she/he is: (a) the representative of the UN in-country and mandated to support the host country achieve its sustainable development (and fund-raising) goals; (b) the Resident Coordinator of the UN family responsible for ensuring that UN programming is coherent and supportive of government; and finally (c) the Resident Representative of UNDP responsible for advancing UNDP's interests and programming opportunities in that country. This was very evident at the International Donor Conference, where job one (and many would argue the only job) was to support the affected governments to make a persuasive pitch to their development donors, and UNDP did not use this gathering of the donors to seek funding for its own projects.

⁵ Within the Regional Bureau for Africa, most policy-related capacity and country office support functions were moved from New York to Addis Ababa. The Ebola crisis hit during this transition and before the capacity in Addis was built up.

⁶ Risks were managed through explicit consideration at the planning stage in the form of risk registers, and risk management behaviour (assessing risks, ranking them, and accepting or mitigating them) in the Crisis Board and within the Bureaux and Country Offices. The empowerment of mid-level officers to take risks was important for success.

COORDINATION AND DECISION-MAKING

KEY COORDINATION ELEMENTS OF THE EBOLA RESPONSE

6. Already in the second quarter of 2014, each of the country offices in the three most affected countries obtained some quick-response additional funding from Headquarters to address an emerging health crisis. At this early stage, WHO was coordinating an initial health response, and together with OCHA and under the leadership of the Senior UN Coordinator on Ebola, developed the first comprehensive assessment called the Overview of Needs and Requirements. Through mid-2014, each of the three RCs coordinated information-gathering and country-level responses through the UNCT, but coordination was kept low profile as the host governments (to different extents) downplayed the scope of the crisis. In Liberia, local coordination got started early with the supporting structure of a pre-existing peace-keeping mission under the authority of a Deputy SRSG, and only in Liberia were elements of the humanitarian cluster system activated.⁷

7. ACTIVATION OF THE CRISIS BOARD

In Headquarters, the Crisis Board (CB) was activated (as Level 2 initially) in late August 2014, and as can be seen in the timeline there was a quick succession of key meetings, policy decisions and initial planning actions, including criticality assessments to reallocate country programme resources, and the development of SURGE plans for each country. As soon as UNMEER was approved by the General Assembly on 19 September (together with a voluntary contributions funding mechanism under the Multi-Partner Trust Fund (MPTF)), UNDP appointed a Senior Coordinator under the authority of the Associate Administrator – in anticipation of a pan-Agency response. Thus it is that, by the end of September, the basic coordination building blocks were in place: global coordination under UNMEER, UNDP coordination under the Crisis Board chaired by the Associate Administrator, supported by a Senior Coordinator, and informed by a SURGE plan that covered the initial scale-up of resources, particularly to the country offices.

MAIN STRENGTHS OF UNDP'S COORDINATION

8. COORDINATION WITH HOST GOVERNMENTS

The online survey revealed a widespread conviction that **coordination with host governments was strong**. There are perhaps three reasons for this. First of all, coordinating with the host government is in the DNA of RCs, and this was not disrupted by the nature of the crisis. Secondly, related to this, when UNMEER mobilised in each of the three priority countries they came in with limited country knowledge or operational capacity, and needed to rely upon the UN agencies on the ground as intermediaries to establish their relationships with host governments. And finally, when the dust had settled on the emergency response and attention turned to recovery, it was UNDP that was still present with the Ministers of Finance and Planning, ready to work with their counterparts on analysis, planning and programming with the host government at the centre. From a host government point of

“UNDP took advantage of its already-existing inter-governmental relationships and networks, across development sectors to consolidate and engage ALL parts of governments. They helped with policy discussions and conclusions during the evolution and development of the governments’ role in Ebola response and recovery.” Review respondent.

⁷ It has been argued in academic analysis of the Ebola response and also within UNDP that a classic humanitarian response might have been more effective in the short term, but would have worked around not through host governments and would thereby have made the transition from externally-led emergency response to nationally-led recovery more difficult.

view, UNDP might not have brought the most resources, but UNDP was a steady government partner for coordination and planning before, during and after the crisis.

9. FIRST TURNING POINT: A CLEAR PURPOSE EMERGED FROM THE ACCRA COORDINATION MEETING

In late September and early October, while the SURGE plans were being put into motion and the Crisis Board was meeting, there was **still a sense that UNDP did not have a clear direction**. It was evident after the creation of UNMEER what UNDP was not supposed to do, but it was not clear in this 'health emergency' what UNDP should do.⁸

*"The vehicle was ready but we were not sure who was driving or where to go."
Review respondent.*

There were advocates for health programming, for survivor benefits, and for early recovery – but none of these provided a satisfactory system-wide rationale. The first of two turning points came on 15 October, when the UNMEER planning meeting in Accra agreed on a clear division of labour between UN agencies. UNDP confirmed its agreement to tackle the essential problem of Ebola worker payments, and immediately thereafter the Agency's management, staff and funding aligned behind this clear purpose. UNDP's first (ever) Development Solutions Team (DST) was formed, and implemented an initiative that is universally regarded as a success. The point to make here is not so much about the Ebola payments project itself, but about **the organizing and motivating effect of a clear purpose**.⁹

10. SECOND TURNING POINT: THE SENIOR HQ MISSION TO THE FIELD

The second turning point in terms of coordination came in late October shortly after the Accra meeting, when three senior managers from Headquarters (representing BPPS, RBA and BERA) visited the three affected countries – more than a touch-down, and including field visits to the front lines. This visit had multiple benefits and is seen from the field perspective as the moment when their morale turned around, when *"HQ finally understood what we were dealing with,"* when HQ was first perceived as responsive to field priorities. **It was this visit that finally built a bridge between two solitudes**, as hitherto HQ had been focused more on its New York relationships with the Executive Office of the SG, UNMEER and with UN member states, and the Country Offices (COs) had been looking in the other direction at operations and delivery. Starting with this visit, HQ and COs understood and started to focus upon each other's needs, and developed a common purpose.

11. OPERATIONAL COORDINATION

Operational coordination was perceived as strong: in particular the working-level relationships between BMS and the other bureaux on very delicate issues of duty of care, medical insurance, emergency recruitment and accelerated procurement. It was felt that **having a single focal point in the office of the BMS Director handling all emergency-related problems was particularly effective**.

12. HARNESSING OTHER PROGRAMMES

While programmatic coordination encountered more difficulties (as we shall see later), there were notable instances where **UNDP was able to access and leverage resources from other programmes hosted by or affiliated with UNDP** and apply them to the Ebola response. For example the re-allocation of GFATM and UNAIDS resources to support Ebola-programming was highlighted, as well as the relationship with UNCDF that was instrumental in the Ebola worker payment project.

⁸ This view is not universally held and some stakeholders have expressed a different view.

⁹ Similar "rallying around a purpose" experiences were reported from the field after country-level meetings in July and August to determine locally how the country offices should respond.

13. RISK MANAGEMENT

Risk management was a constant theme at all levels of the response, and on balance **risks seem to have been managed rather well**.

Technically, risks were properly assessed in the SURGE planning (although not monitored during implementation), and the risks of major decisions were explicitly considered by the Crisis Board.

Ensuring that senior management was fully and frankly informed of issues and therefore equipped to assess risks was a key role played by a cohort of mid-level officers and most definitely facilitated decision-making on heavy issues of staff health and safety, financial management and organizational reputation. On the whole, risk-takers enabled a quicker and more effective response, whereas there were some reported instances where risk-avoidance slowed things down, particularly in the field.

"Let your senior people take risks and make decisions without fear and with purpose; clear the path so decisions can be turned into actions while maintaining accountability."
Review respondent.

14. WORKING ON WHAT YOU KNOW AND IN WAYS YOU KNOW

On the whole, UNDP staff in all locations felt that UNDP's coordination work was **more effective when UNDP was working within its established mechanisms (UNCT) and on issues that are within UNDP's normal experience** (i.e. governance, capacity-building,

supporting national coordinating machinery, national planning), and

less effective when working through ad hoc mechanisms. It also seems likely that the reason some proposed activities were unfunded is because donors could not see UNDP's comparative advantage in those areas. At the same time, the valuable work on Ebola worker payments illustrates the importance of allowing UNDP to "think and work outside the box" and tackle issues that are in UNDP's capacity and experience, but that some donors and observers might not see as a core part of UNDP's business.¹⁰

"Donors will not fund you for something that they think is someone else's mandate."
Review respondent.

MAIN CHALLENGES AND GAPS IN UNDP'S COORDINATION OF THE EBOLA RESPONSE

15. COORDINATION WITH UNMEER

Not surprisingly, the online survey revealed that **the weakest dimension of coordination was with UNMEER**. The AAR will not dwell on this point, but notes that UNMEER's own AAR endorses this perception, and also signaled the value of working with (not around) existing structures, and the shortcomings of a one-size-fits-all approach that did not sufficiently adjust to the very different institutional contexts of the three most affected countries.

16. CRISIS BOARD

Crisis Board was an effective mechanism, but with two shortcomings that were observed mainly by respondents outside the Crisis Board (staff at the working level in HQs and in the field). One shortcoming was that, for whatever reason (and no-one thinks this was an easy task), **Crisis Board was not able to provide the early and clearly articulated statement of UNDP's role and key priorities that was needed** to align the bureaux and the field around a common purpose.¹¹ As mentioned earlier, in the absence of this shared vision and common purpose, the willingness and energy of many staff was not as focused and effective as it could have been particularly in the first

¹⁰ In discussion with UNDP staff, UNDP's mandate was helpfully characterised not by sector, but as developing the capacity of key government and non-government institutions.

¹¹ The challenges of developing the 'overarching narrative' are discussed in more depth later in this review, and these challenges were shared with many major multi-sectoral organisations including UNMEER. The field visit by a team of senior management was key to building a shared vision (as mentioned earlier).

two months, and programming was difficult to frame without this strategic direction and context.¹²

The second observation noted by many is that over time, **Crisis Board became formulaic, it lost its decision-making sharpness and became a body for exchange of information** that could have been available to more people and in real time if a stronger Information Management mechanism were in place in CRU. For a while, the field felt that Crisis Board was more of a burden than a support. Finally, the Board was so large, and attendance of senior officials waned over time, to the point that many key management decisions ended up being made with the authority of the Board but offline in small issue-specific meetings of senior managers.

17. CRISIS RESPONSE UNIT

The Crisis Response Unit was widely credited with having managed well and in particular the deployments, but some respondents felt that **CRU could have played a stronger role in supporting the work of the Crisis Board** with good preparation, useful documentation, clear agendas and prompt meeting reports. Some of those with field experience of humanitarian operations coordinated by an HC/HCT wished that CRU had stronger and more systematic coordination and information management capacities (see further discussion below).

18. PROGRAMMING COORDINATION

Programming encountered many challenges, although there were clearly programming successes - notably the Ebola Worker Payment project and several country-level initiatives financed by the MPTF. Interviewees offered many possible reasons for this, and in the end we conclude that **programming was victim of several factors largely beyond the control of the people involved**.

Among these we can suggest (a) the genuine **confusion resulting from the reorganisation** (initially it was unclear who was responsible for taking the lead on programming: RBA, CO or BPPS, and if BBPS then which Division?), compounded by the fact that (b) the **goalposts kept moving** as programming attention shifted rapidly between thematic areas (from health to cash transfers to recovery – which is itself already a difficult area for programming); (c) the **initial absence of a clear corporate vision** about what UNDP should be doing in the Ebola response, given that the situation did not obviously lend itself to UNDP's comparative advantages and mandate, and that UNMEER had been mandated to coordinate essential services; (d) problems with **field-HQs communications** such that staff could be working on similar issues on either side of the Atlantic but in isolation of each other -- and possibly heading in different directions; (e) the **untested nature of the brand new mechanism** designed to permit cross-thematic coordination on priority issues, the DST¹³, which at that point was able to handle a single clear mandate-aligned task (Ebola worker payments), but which was not yet established enough to take on the multi-themed complexity of designing a comprehensive regional approach to recovery¹⁴; and (f) **lukewarm donor interest** in funding UNDP's response and recovery work in the three most-affected countries at the time of the crisis (see later discussion on resource mobilization).

The purpose of the DST is to provide, "effective and coordinated technical and programming support to Country Offices." Review respondent.

19. REGIONAL COORDINATION

Finally, regional coordination was also regarded as weak: from two perspectives. Firstly, the **Regional Offices were in full transition mode** as HQ was scaling back staff in favour of scaling-up in Addis, and transferring resources from Dakar and Johannesburg to Addis. Dakar's role in coordinating regional economic analysis was appreciated, but this was not enough. UNDP's management model assumes that the Regional Service Centre would be

¹² This view is not universally held and some stakeholders have expressed a different view.

¹³ Initially the Early Recovery DST had 11 thematic areas plus two additional ones reporting to the CMT, and team members in six locations.

¹⁴ This view is not universally held and some stakeholders have expressed a different view.

equipped to step in and play a strong role in coordination and support – but Addis was not ready to do so for Ebola. Secondly, there was, and even today there still is, **no practical mechanism for coordinating the work of three contiguous country offices** who share a common transboundary problem.

KEY RECOMMENDATIONS TO UNDP FOR COORDINATING IN A MAJOR HEALTH CRISIS

20. APPOINT A SENIOR COORDINATOR AT THE OUTSET

At the top of the recommendation list is **appointing a senior coordinator for the response**. Coordinator attributes might include: understanding UNDP and the UN Secretariat; people skills; leadership skills; the ability to speak truth to power; and a can-do problem-solving attitude. Note that it is not assumed that this senior coordinator role would automatically be an ad hoc office outside the normal UNDP management structure (as was the case for Ebola). Depending on the context, this could simply be the designation of an existing ASG or Director as the ‘Senior Coordinator’, with accompanying special authorities and responsibilities that would last as long as the crisis is considered to require these special measures (presumably while L3).

This **senior coordinator should have the authority and resources required to implement the mandate**, which ideally includes a small staff team, and access either directly or through CRU to a quick-disbursing source of flexible funding that will allow Country Offices to engage early, strategically and confidently, and thereby help secure UNDP’s leadership credibility in the field.

21. ENSURE FIELD LEADERSHIP IS READY

The first actions of senior management should be to **assess the leadership configuration in the field**, to assign a senior executive to support the RC if necessary, and to provide immediate SURGE support in areas that are inevitably going to be needed in a major crisis (see recommendations for SURGE below). The **working assumption should be that the existing structures need reinforcing**, not that they should be replaced with an ad hoc structure. The goal is to be strong enough, flexible enough and quick enough that an UNMEER is not needed in a future health emergency.

“[UNMEER] might not have been required if the UN system had come together sooner, with stronger HC-type support at a country-level and a greater willingness to step outside the usual programming constraints earlier in the response.” Review respondent.

22. DETERMINE UNDP’S ROLE

Based on field knowledge and corporate analysis, **determine what is the appropriate role for UNDP to play**, taking into account other actors and UNDP’s comparative advantages, and develop a **clear statement of goals and objectives** around which the entire organisation can rally and organise.¹⁵ This clear vision and the role and authorities of the coordinator should be **communicated confidently by the Administrator** to all.

Where the coordinator should sit within the organisation depends upon the nature of the crisis. Ideally the **coordinator should be positioned close to senior management and close to the operations**, which in most cases would suggest attachment¹⁶ to the Regional Bureau responsible for the affected country – thereby reducing transaction costs and increasing the likelihood of alignment between the HQs response and the priorities of Country Offices. If the crisis is global or requires coordination across the Agency, then the coordinator could be

¹⁵ With the experience of Ebola, it would today be easier to craft a message around a medical crisis being the consequence of governance and market failures – but that business case was not evident at the outset of the Ebola crisis.

¹⁶ Note that the senior coordinator might not be a new position but could also be the assignment of special authorities and a support team to an existing position in the organisational structure.

attached to the Executive Office or to CRU.

23. IDENTIFY THE KEY FOCAL POINTS

Wherever the coordinator sits, s/he should be well supported and **each Bureau should identify a key focal point for the emergency** – a single gateway for communications, information and tasking so that staff do not receive multiple requests for similar information through different channels.¹⁷ It was a feature of the Ebola response that **organizational awareness and a can-do attitude were more important qualities in key staff than seniority**, and there were many examples of relatively junior staff in these focal point roles proving themselves effective because they were trusted to manage risks and empowered to solve problems.

“In an emergency you need problem-solvers not problem-makers. The decision will get made one way or the other, but the work will get done quicker and with goodwill if the key people have a problem-solving attitude.” Review respondent.

Crisis Board should meet as needed to make Agency-wide decisions, but should not meet only for information sharing. This suggests (depending on the crisis) that the frequency of meetings would reduce over time as the focus shifts from setting up to implementation, and that participation would similarly migrate from senior managers to operational managers.¹⁸

24. ENHANCE CRU WITH SOME INFORMATION MANAGEMENT AND COORDINATION CAPACITY

The Crisis Response Unit should enhance its management tools and skills, in particular a stronger capacity for Information Management (ideally a centralized universally accessible real-time repository of key documents and reports); as well as capacity to support the CB manage agendas, keep meetings focused, decisive, and relevant to who is in the room.¹⁹ Contextual factors might determine whether CRU provides these services to the CB, or whether CRU should temporarily deploy support for IM and CB support to the Bureau that is chairing the Crisis Board.

Assuming that there is no UNMEER-type mechanism and that field coordination will be conducted through UNCT/HCT machinery as appropriate, it is recommended that there be an **early orientation of UNCT members on the coordination mechanism, roles and expectations, so all UNCT members are on the same page** when it comes to messaging. In some cases it might be preferable to expand the UNCT to a UNCT plus, involving key non-UN partners especially strong INGOs. Co-locating the supplemental response teams in one facility (as was done with the Special Court in Sierra Leone) is particularly effective for enhancing coordination and coherence.

25. SEND A HIGH-LEVEL MISSION TO THE FIELD

Sending a team of senior staff to the field early (the Senior Coordinator and key ASGs) will facilitate (a) a clear strategy with common messaging, (b) risk assessment, (c) assessment of field capacity and priority staffing gaps, and (d) consolidating an initial SURGE response plan. The precise composition, timing and mandate of a senior mission to the field should be determined on a case-by-case basis depending on the context.

¹⁷ Similarly, it is recommended that there be a single focal point for external communications, and another for relations with the operational units of the Secretariat such as Medical Services and Security.

¹⁸ Whether the response is considered L3 or L2 should not depend upon the seniority of CB participants so much as the need for the special authorities and mechanisms that might be afforded by L3 status.

¹⁹ The extent to which these would be needed in any particular crisis will depend also upon OCHA's role: CRU should not be duplicating OCHA, but there could be cases (such as Ebola) where OCHA is not mobilised, and UNDP has a need to coordinate its own information that does not belong in a public space.

DEPLOYMENT MECHANISMS

KEY CHARACTERISTICS OF DEPLOYMENTS IN THE EBOLA RESPONSE

26. DEPLOYMENT PLANNING

A SURGE planning team leader was appointed and an Ebola SURGE Plan was prepared in September 2014, after the Crisis Board had been activated and at the same time as UNMEER was created. The SURGE plan was conceptually framed by three very fresh and not entirely joined-up strategic documents: WHO's Ebola Response Roadmap, OCHA's Overview of Needs and Requirements, and the five Strategic Objectives of UNMEER. In that already muddled context, the SURGE planning team leader worked with the three country offices and compiled their inputs, combined them with an assessment of HQs needs, and produced on 3 October a full SURGE Plan with a requested budget of \$12.7 million split between two phases of 0-3 and 4-6 months.

27. DEPLOYMENT OPERATIONS

Even as the plan was being drafted, an initial handful of early recovery, health and communications experts were deployed on extended missions to the affected countries. These were followed in November by a first wave of SURGE deployees, and thereafter deployees were extended or rotated until July 2015. Altogether, 63 separate deployments were made (including a few cases of double deployments): 21 to Liberia, 16 to Sierra Leone, 14 to Guinea and 12 to Accra or to roving officer positions. Initially deployees were a blend of staff ("true" SURGE) temporary appointments and individual consultants, but in order to provide full protection to deployees in a situation of uncertainty with regards to medical insurance and medical evacuations, all consultant deployees were moved onto temporary appointment contracts.

MAIN STRENGTHS OF UNDP'S DEPLOYMENTS

28. THE SURGE PLAN

The online survey, interviews and document analysis all confirm that the **SURGE Plan was a strong document, and an example of HQs and the field being well-connected** -- primarily because the Plan was field-led and had the explicit bridging purpose of identifying field priorities to HQs. The Plan is notable as an Agency-wide cooperative effort right at the moment of organisational transition, for the way that it handled risks, and also for the detail and accuracy of its predictions of the skills that would need to be deployed. Without such an early and detailed plan, UNDP's response would certainly have suffered, and the fact that the programmatic elements of the SURGE plan were quickly superseded by other strategies and plans should not detract from this conclusion.

29. DUTY OF CARE

Duty of care was a huge preoccupation and was seen as well-managed. At one level the boundaries of UNDP's responsibility were clear -- the main problem was the risk that UNDP would not be able to fulfil its duty (i.e. evacuation to a safe treatment facility in the event of a staff-member contracting Ebola). A second problem was the reluctance of staff on the roster to deploy given the perceived exceptional personal risks of moving into an epidemic zone and the potential risk of transmission to family members, compounded by the need to spend 21 days in post-country isolation before resuming normal life. UNDP took exceptional measures to provide a danger pay supplement, to arrange for evacuation, and to strengthen the UN clinic in each country -- not to provide in-country capacity for Ebola treatment, but rather to ensure that staff and families of the UN of system and the broader aid community of embassies and NGOs were able to access quality health care in Ebola-free clinics. The

benefits of this went way beyond UNDP staff, and this is only one of many examples where UNDP provides common services that are often taken for granted and for which the full costs are not recovered from partners. A second aspect of duty of care, highlighted by field respondents, were the small but significant special measures taken by the CDs in each country to immediately enhance the personal safety of their staff and their families, not only in capitals but also in field sites. Finally, the fielding of staff counsellors was appreciated.

30. SOME SUCCESS FACTORS

There were some success factors for deployments. First of all, many of the more **successful deployees were found through networks** not through the roster. Some Country Directors sought out people they already knew, already trusted, and prevailed upon them to overlook the risks and mobilise quickly. In other cases, very specialized skills were needed (i.e. payments) and that were not well-represented on the roster. Secondly, **UNVs turned out to be among the most cost-effective** deployees, and even though they were not part of the regular SURGE mechanism (for example they do not figure in the CRU deployee statistics provided earlier) they filled key positions in particular in the provincial and local coordination units in Sierra Leone, and in the Government coordination unit in Guinée. Third, the **flexibility and cooperation of BMS** were essential to the success of the deployments, as there were operational issues of all types that needed a thorough understanding of how to apply the spirit of emergency without breaking the letter of the rules. Having a single person within BMS for coordination of all Ebola-related human resources matters was particularly helpful. Finally, it was widely felt that **UNDP staff were more effective than consultants** especially if this was their first experience with UNDP. People from the inside or who had previously been staff knew the culture and systems of the organisation, were more autonomous, more flexible and ultimately more effective.

“In a crisis we need more than good intentions - we need experienced professionals who are ready to take decisions.” Review respondent.

MAIN CHALLENGES AND GAPS IN UNDP’S DEPLOYMENTS DURING THE EBOLA RESPONSE

31. The online survey generally viewed the process of deployments as well-performing, but there was a scoring dip with regard to “people being in the right place at the right time.” It seems from the interviews that while there was some concern about the ‘right place’ (notably whether deployees to Accra were best able to support the country offices), the main issue was the ‘right timing’ of deployments, and particularly the delay of 4-6 weeks in the deployment of the first batch of external recruits, during which time contracting modalities, insurance and evacuation arrangements were being worked out. All UN agencies experienced similar challenges at this time.

32. SLOW DEPLOYMENTS

In practice there were three waves of deployments. The first wave in September took place even before the SURGE Plan was in place. It was made up exclusively of staff deployed on mission status: they were quick and effective, and most of them had left by early October. Then **there was a hiatus of a month with few or in some cases no deployees**: this was a period of recruitment of both consultants and staff to fill the positions approved in the SURGE Plan. This recruitment period was characterised by three main problems: (a) severe delays – sometimes as long as six weeks – in obtaining medical clearances for consultants from the ExpRes roster,²⁰ (b) difficulty in obtaining supervisor approval for the release of staff who were already in the SURGE roster, and (c) a major effort to canvas the entire organisation for volunteers – an effort launched with the best intentions outside the SURGE roster system, and which ended up creating expectations, consuming a lot of management and staff energy, but resulted in no deployments.

²⁰ Additional staff were seconded to expedite these processes and without whom the delays could have been longer still.

This delay turned out to be very costly to UNDP, because **the first people to arrive in the field in an emergency situation have a huge advantage in terms of access, knowledge and strategic positioning**, and the fact that UNDP's emergency deployees were absent at precisely the moment when the interagency systems were being set up and roles were being assigned,²¹ meant that in November **UNDP's deployees started with a strategic disadvantage and needed to focus on catch-up** and finding a place to fit in. After exceptional efforts in HQs, the main administrative blockages were overcome (including the resolution of medical insurance) and the second wave, the first of "true" SURGE deployees, took place in November 2014. The third wave, SURGE 2, took place in March 2015 when most of the first wave were replaced at the end of their initial assignments.

33. ENABLING CONDITIONS FOR DEPLOYEES

There is one other sense in which the right people might not have been in the right place at the right time: some deployees who were capable were not able to be fully effective because **the enabling conditions for their success were not in place**. There were three examples of this. The first is that when a Country Office is facing a crisis and is chronically short-staffed, it is to be expected (and should perhaps be anticipated by CRU) that **the first person to arrive on the ground is going to get assigned to the Country Director's most important priority** – regardless of whether this is the function for which the deployee was recruited. A second example was the first batch of early recovery advisers, who were deployed early for all the right policy and programmatic reasons, but who were unable to engage either the host governments or partners (and least of all UNMEER) in useful early recovery discussions or planning, because the collective mindset was so firmly fixed on the "Stop and Treat" components of the interagency response. And finally, the communications advisers who were fielded were universally praised for their abilities and responsiveness, but they were not equipped to follow through their communications work with donor engagement, so the connection to resource mobilisation was not made.

34. SUPPORT FOR DEPLOYMENTS

As has been mentioned earlier, on the whole staff deployees were quicker to fit in and better equipped to navigate the ambiguities of a crisis situation. Consultant deployees (and some staff) expressed the need for support of four kinds. First of all, there were some **deployees who would have benefitted from a more systematic induction**. We heard of deployees who were not briefed on arrival, were not given a basic orientation to who within the UNDP office is doing what, and were not given clear objectives. An arrival briefing, a summary organisation chart and snapshot of roles would have helped them get launched quicker. Secondly, some **small country offices did not have enough operations resources** and were less able to provide deployees with the material support they needed to be effective, in particular office space, laptops, and access to vehicles.

Third, there were some deployees who felt that they were deployed and then left to fend for themselves. One deployee expressed the view that, while the deployee had an immediate supervisor in the field, s/he felt the need for coaching support and guidance from someone who was the custodian of the whole deployment – from someone who could troubleshoot, provide mentorship, and who s/he could bounce ideas off. This concept is difficult to capture in a nutshell, but we can perhaps summarise it as **each deployment needing an 'owner' who understands and supports the deployment through to its conclusion**. In some cases, this could be the immediate supervisor in country, but for some deployees (particularly technical experts or staff with a function that is not normal within the Country Office) this 'owner' could be in a Regional Office or in HQs.²²

Finally, there were **administrative and procurement bottlenecks** in the field that prevented some programming deployees from getting their work done on time. We did not have the time or the mandate in this AAR to look in detail at technical and procedural problems, but we did hear different points of view. Some interlocutors felt that the field procurement staff had not "switched into emergency mode" and were either not prioritising the Ebola response components, or were not applying the more flexible rules that are permitted in emergencies -- either

²¹ It is important to note that UNDP's Country Office teams remained *in situ* throughout the crisis.

²² It was suggested that UNV has deployee support mechanisms that could provide useful examples.

through lack of understanding of the rules or unwillingness to take risks that might result in a career-threatening audit observation. Others felt that the fast track rules are not sufficient – that they accelerate the administrative processes but do not actually simplify them, and instead create pressures and expectations that are frustrated because the fundamental bottlenecks have not been addressed.

35. LACK OF INTEGRATION OF SURGE WITHIN THE COUNTRY OFFICE

It is a common problem in humanitarian situations that **emergency employees work in parallel to the permanent field staff**, and this was also the case with UNDP's Ebola response -- although the level of integration varied across the different Country Offices and depended upon the functions of the employee. Lack of integration at country level can have four effects that we saw also in UNDP's Ebola response: (a) first, some "permanent" field staff can be demoralised if they feel displaced or marginalised; (b) secondly there are missed opportunities for optimising local knowledge in the emergency; (c) third, the separation of emergency from regular programming inhibits linkages between the two and stifles early recovery thinking; and finally (d) the understandable tendency of emergency employees to relate more to HQs (and in this case to UNMEER) than to the Country Office/host government stands in the way of developing a single, clear and coherent strategy.

But this is not a straightforward matter. Integrating with a Country Office carries the transaction costs of system adjustment, and some degree of separation has its benefits as it allows the emergency team itself to get up and running quicker, to be more focused and internally coherent. We heard SURGE employees who were in full 'emergency mode' argue persuasively that the SURGE team leader should attend UNCT meetings, and that some employees should be assigned to/located within UNMEER. In the end we conclude that **the way in which the SURGE team is situated in relation to a Country Office should be determined by the specifics of the emergency and the context on the ground** – it is unlikely that one-size-fits-all here. But we do believe that there is an important overarching principle that should be followed, and that is that the **Country Office should always be in the lead and that the SURGE's primary purpose is to support the Country Office**. The recommendations below follow this premise.

36. WRAPPING UP AND HANDING OVER AFTER THE SURGE

In an emergency and especially at the start of an emergency, it is understandable that the major effort should go into the deployment process. But **the effectiveness of deployments also needs attention to the stages of wrap-up, handover and disengagement**. There was a continuous dialogue between CRU and the field on deployments, but the feeling in the Country Offices was that they were not sufficiently involved in key decisions about terminations and extensions, and therefore were not able to optimise the available resources and best match them to the evolving needs of the field. The review concludes that the gap between CRU and CO perceptions on this matter is probably more a gap in communications and expectations than a problem of mechanisms.

Handover between employees was done as best as possible, but as noted earlier there was a complete hiatus between the initial mission employees and the first wave of "true" SURGE employees, and thereafter **there was not always an opportunity for overlap between employees in the same portfolio, there were not always systematic handover notes**, and in general there were few end-of-assignment reports. Some employees also found that the administration of their separation was overly cumbersome, including problems of late payment.

Country Offices were generally under-resourced before the crisis, with the result that **some employees, especially operations staff, ended up at least partly filling chronic gaps within the Country Office**, and the Country Offices had difficulty letting go of this much-welcomed boost. Similarly, the Country Offices remained under-resourced after the crisis: as discussed later, resource mobilisation outcomes were not at levels hoped, despite extensive efforts at all levels, and as a consequence the COs were not able to follow the crisis response staffing 'model' which suggests that Country Offices should regularise CRU-provided SURGE support after the initial three months.

KEY RECOMMENDATIONS TO UNDP FOR DEPLOYMENTS TO A MAJOR HEALTH CRISIS

RECOMMENDATIONS TO CRU FOR MANAGING DEPLOYMENTS FROM HQ

37. CRU should consider strengthening itself in two areas. One is to **obtain Information Management capability**, which might include real-time online information on the evolving situation, on the location and functions of every deployee, key policy and assessment documents, agendas and minutes of coordination meetings, and even real-time tracking of funding for both the response and early recovery.²³ The second is the establishment²⁴ of a quick-response programme funding mechanism – possibly a small corporate reserve that would be replenished by donors after each time it is activated – and that would allow the field to immediately finance small strategic initiatives that fill critical gaps and secure UNDP’s responsive credibility.

38. In terms of its existing infrastructure, CRU should keep working on its rosters, on updating its SOPs, and on improving its tools. Recognising that staff deployees were generally viewed as more effective and adaptable than consultants, it is important that the SURGE deployee roster be kept fresh, and that **supervisors are clearly reminded by senior management that a Level 3 crisis is a corporate priority and requires full cooperation with releasing deployees in 24-72 hours**. In some cases the deployees on the ExpRes roster were found not to have experience commensurate with field expectations, so there would be value in deepening the due diligence on the roster and ensuring that candidates have performed effectively in comparable crisis situations. It was also felt that some deployees had narrow skillsets and were not as able to work horizontally – in a team rather than on a specific narrow issue: so **soft skills and teamwork should be priority selection criteria for deployees**.

39. In terms of tools, CRU is encouraged to **keep developing its Crisis Response Packages** that facilitate quick and consistent implementation of core functions while remaining sensitive to context, and that include resource mobilisation guidelines tailored to the specific crisis component. If the Fast Track Procedures still aren’t considered “fast enough” by Cos, then there would be justification for development of a Fast Track 2.0, in particular with regards to Human Resources and Procurement.

40. When crisis strikes, and here it is important to recall that we are considering a large-scale sudden crisis that triggers L3 and a corporate response, CRU and BMS can predict that staffing is going to be a major initial pressure, and (as was done for Ebola) should designate a dedicated human resources professional to support the deployments, as well as boosting the capacity of United Nations Medical Services to expedite medical clearances. **The priority for immediate deployment should be staff rather than consultants**, and several respondents recommended that these ‘first responders’ should include IM, procurement, finance and human resources support to the affected Country Office, bearing in mind that a UNDP Country Office not only provides support to UNDP but **also provides core support services to other UN agencies who will also be scaling up their responses** and loading up their operational needs.

41. Deploying a few core staff should be done immediately, as the first people on the ground will have important advantages in terms of knowledge, relationships, access and strategic positioning, and will greatly increase the prospects of UNDP being able to occupy and perform in the coordination space. Also, just as an emergency needs to consider early recovery from the outset, **emergency deployments need to consider transition from day one**, always thinking ahead to how they will connect up with long-term functions, positions and programmes: for example not creating processes and systems that cannot be sustained in normal circumstances and that would collapse

“You need to be present in the crisis in order to be credible and effective leading after the crisis.”
Review respondent.

²³ Note that we are not recommending that CRU take over emergency-wide information management functions that might be the mandate of OCHA, or analytical work that belongs in the domain of the Regional Bureau – but rather UNDP-wide information needed to support management decision-making and reporting.

²⁴ A new funding window for Emergency Development Response to Crisis and Recovery was established post-Ebola. The AAR’s findings reinforce the need for this mechanism and do not recommend something additional.

with the departure of the deployee.

42. In terms of wrap-up, it is recommended that there be an **explicit exit planning process involving the HQs and field** stakeholders several weeks or even months before the expected end of the emergency phase, in which all could agree on the fine-tuning of the last wave of deployments: which deployees would exit earlier and which later, and how the key functions of deployees would either be closed down or handed over. It is important that the Country Office have a role in co-managing this exit stage. Finally, all deployees should be required to write a debriefing report.

RECOMMENDATIONS TO REGIONAL BUREAUX FOR MANAGING DEPLOYMENTS IN THE FIELD

43. As mentioned earlier, the relationship between the SURGE deployees and the Country Office should allow for variation according to context, but in general it is desirable for **SURGE deployees to be seen and managed as a support to the Country Office**, preferably reporting to the appropriate Deputy Country Director and integrated into programing and operations systems as appropriate. This will also enhance synergies between the emergency programmes and regular programmes. Related to this, it should be the responsibility of the CRU to **ensure that every deployee has a clear ‘owner’ of the assignment** – a go-to person to troubleshoot the deployee’s problems, to confirm the mandate, to validate work accomplished and to provide ongoing support (this could be a supervisor but there could be circumstances where the supervisor is not the best placed to provide this sort of support).

UNDP'S CONTRIBUTIONS TO THE RESPONSE

UNDP'S KEY CONTRIBUTIONS TO THE EBOLA RESPONSE

44. UNDP'S ANALYSIS AND COORDINATION CONTRIBUTIONS

UNDP's contributions to the Ebola response were many and varied. Coordination contributions can be divided between four distinct phases. First was the early stage of response up to the creation of UNMEER – a period when all the UNCTs were struggling in their different ways to understand the scope of the Ebola epidemic and how they could best support their host governments (which in some cases were reluctant to recognise the severity of the crisis). Initially the consensus was that WHO should lead the response, which at that stage was still seen as primarily medical, although later in August and September there was a brief discussion about whether this was a humanitarian emergency warranting coordination by OCHA. Only in Liberia, where there was an established humanitarian machinery and a UN peacekeeping mission UNMIL headed by an SRSG, was the humanitarian machinery activated with a conventional HCT and (adapted) cluster structure.

From the moment UNMEER was created the overall coordination was taken over by UNMEER, although as pointed out earlier, **UNMEER did not fully implement the essential services pillar of its mandate and thus early recovery was left in coordination limbo** for several critical months. During this second stage, UNDP's coordination work was mainly internal, including an early attempt (end November 2014) to raise funding from donors for UNDP's response and recovery work.

At the end of December 2014 the third stage began when UNDP was formally tasked with addressing early recovery, and immediately launched a series of coordination activities including country analyses and an Ebola Recovery Assessment, leading up to the Donor Conference of July 2015, which coincided with the end of the UNMEER mandate. The final phase, beyond the scope of this AAR but still ongoing, was the post-UNMEER period of resuming the normal work of UNCTs and putting in place programming to support host country recovery, including the transfer to UNDP of some UNMEER-initiated MPTF projects as well as UNDP's new programmatic contributions.

45. UNDP'S PROGRAMMATIC CONTRIBUTIONS

UNDP's programming support was provided during all four of the phases described above and were of two basic types: support for the evolving programming priorities of the host countries (i.e. helping host governments and the Mano River Union prepare and present their own multi-year strategies and related programmes), and UNDP's own strategies and programming in support of the affected governments (through reallocated existing resources, new internal funding and new external funding mainly from the MPTF). It is precisely because UNDP was working at the same time on both the response and the early recovery aspects of the crisis, in support of host governments and on behalf of UNDP itself, and in three countries as well as at a regional level, that it was difficult to pin down UNDP's role and purpose at any point in time.²⁵

STRONGER CONTRIBUTIONS MADE BY UNDP

46. UNDP's internal work in the Country Offices in the early stage included **programme criticality assessment** and the redirection of a number of existing projects to support the Ebola response. In addition to repurposed GFATM and HIV/AIDS programmes that had obvious public health capacity and resources, there **were several smaller and**

²⁵ This view is not universally held and some stakeholders have expressed a different view.

imaginative initiatives that were successful but not public and splashy – for example the work done by the governance team in Sierra Leone to prevent the introduction of Ebola into the prison system, which had the unintended result of also decongesting prisons by providing due process to many inmates whose incarceration was unwarranted. At the end of this first phase, significant work was invested in the SURGE plan that was discussed in the previous chapter of this AAR.

During the second phase of the UNMEER-coordinated response, UNDP Country Offices were led by the Senior RBA economist in New York to develop a suite of **country-level economic analyses that framed the case for early recovery** and laid the groundwork for the Ebola Recovery Assessment. In at least one instance, Sierra Leone, this early economic modelling also informed national financial planning for 2015 – arguably another unintended result. This was also the stage when UNDP put in place its **two basic building blocks of response: the universally-praised Programme of Payments to Ebola Response Workers (PPERW), and the strengthening of the UN clinics** (discussed earlier in the context of duty of care).

47. CONTAINING EBOLA IN MALI

In late 2014, there was a minor outbreak of Ebola in Mali. In the absence of UNMEER country office, and learning from the experiences of the three most-affected countries, the UN machinery swiftly responded to contain, trace and treat, and successfully stopped Ebola in its tracks. **UNDP played a significant role in supporting the contingency planning, and in coordinating this system-wide response.** Similar contingency planning and heightened preparedness work was conducted in several other countries bordering on the epidemic's epicentre.

"The successes achieved, such as the payments programme...did not come about by accident. They were part of an overall strategy of thinking outside the box, maintaining flexibility to meet evolving knowledge [of the] impact of the crisis, and mobilising all internal expertise, even in areas considered unconventional for UNDP." Review respondent.

48. THE EBOLA WORKER PAYMENT PROJECT

The PPERW was not an easy project to implement in this context of high reputational, fiduciary and duty of care risk, and after hearing the views of several observers we feel that **success factors included: capable and confident leadership, prior relationships with both UNCDF and the World Bank, institution-wide buy-in and support, sufficient resourcing including from UNMEER/MPTF, in part because the PPERW was well aligned with UNDP's comparative advantages** and with the priorities of the Ebola response. It landed right in the success zone and was well-managed. PPERW also provides another example of an unintended result, as the project has been converted into an ongoing programme under the 'Better Than Cash Alliance' to continue to pay government workers electronically.

49. EBOLA RECOVERY ASSESSMENT

During the third 'early recovery' phase of the response there were two bookend events that stood out as successes in both interviews and in the online survey: one was the **Ebola Recovery Assessment (ERA) of January 2015, and the second was the International Donor Conference of July 2015.** The ERA was considered a strategic success in HQs, but was also seen as tactically weak by the country offices.²⁶ Strategically its great benefit was that it brought several development agencies and donors together to collaborate on early recovery (notably the World Bank, African Development Bank and the European Union), and it got early recovery on the international agenda. It marks the **turning point when everyone's attention started to shift from response to recovery** – from humanitarian mode to development. The ERA also started a thread of analysis and strategic planning at global and country levels that can be traced through the main donor conferences starting with the AU Summit, then the EU Brussels meeting, and the World Bank meeting, leading up to the July International Donor Conference (see the timeline). However, from the Country Office viewpoint the ERA had several shortcomings: primarily that was

²⁶ This view is not universally held and some stakeholders have expressed a different view.

driven by the institutional interests of its team members and had little or no host country buy-in – in some ways even competing with country-led processes that were starting up at the same time. The ERA was also produced at breakneck speed in order to meet a Secretary-General deadline of the AU Summit in Addis on 30 January 2015, and under these time pressures it was felt to have methodological loopholes.

50. INTERNATIONAL DONOR CONFERENCE

Similarly, the International Donor Conference of July 2015 was heralded as a remarkable achievement, all the more because throughout its preparation it encountered scepticism from donors, resistance from affected governments, and tepid support from throughout UNDP itself. A fundamental misunderstanding regarding the conference is that it was seen by some as a UNDP event, and many in UNDP regret that there was not enough attention paid to UNDP fundraising.²⁷ In reality **the conference was from the outset a UN event**, more accurately it was the Secretary-General's event, and UNDP was asked to support it in its capacity as lead on Ebola early recovery. It was managed by a separate team brought together and led by the Senior Coordinator, which helped delineate its system-wide objectives from UNDP's own objectives.

Its focus was to raise awareness of country economic impacts and recovery needs, to table coherent albeit ambitious country recovery plans,²⁸ and to invite donors to contribute to the recovery priorities of the countries. In this it succeeded, although in retrospect it seems there was little truly 'new money' for recovery, as most of the pledges turned out to be repurposed funds (some of which had been pledged in another form at earlier donor meetings. Perhaps the most important lasting result of the Conference was the UNDP-supported effort to craft and corral the three country recovery plans and the unwieldy Mano River Union regional plan, as to this day **these plans remain the best expressions of the medium-term recovery strategies of the three affected countries**.

51. FIELD COMMUNICATIONS

Finally, it was remarked by several observers that deploying communications advisers to the field allowed UNDP to capture **images and inside stories that are rarely seen in UNDP work**; there was better UNDP branding and more active social media than usual.

MAIN CHALLENGES AND GAPS IN UNDP'S CONTRIBUTIONS

52. DONORS HAVE DIFFICULTY FUNDING EARLY RECOVERY

Probably the biggest disappointment was the weak partner interest in early recovery. We will discuss some of the programming reasons for this below: the initial point to make here is that the prospects for early recovery funding were always low because of **persistent biases against early recovery stemming from the ways donors plan and manage their development assistance**. For the most part, donors segregate their humanitarian from their development spending. But because their humanitarian mechanisms cannot usually fund 'development' work (including early recovery), and their development funding is planned several years in advance, donors have a mechanical problem with releasing significant amounts of funding at short notice for

*"Early recovery is as much political as technical – you need to navigate host country relations, interagency issues and internal dynamics."
Review respondent.*

²⁷ The UNRC and UNDP were heavily involved in supporting the government to develop their national strategies ahead of the conference. The idea was that UNDP would capitalize on this partnership to design projects that responded to both the priorities as set out in the national recovery plans and to the donor pledges. This was not fully realized after the conference.

²⁸ The plans might have been more realistic and more coherent if the ERA had been more country led such that there was an unbroken chain of country-led planning from the ERA through to the Conference. This view is not universally held and some stakeholders have expressed a different view.

early recovery.²⁹ Add to this that each donor tends to focus on a few countries (Guinea being one of those with very few donors), and also that donors do not generally entrust their funding to governments viewed as corrupt, and it is understandable that donors would prefer to reallocate or recycle their existing commitments rather than bring significant new development funding to the governments of the affected countries.³⁰

53. UNDP WAS DISCOURAGED FROM SEEKING EARLY RECOVERY FINDING AT THE HEIGHT OF THE RESPONSE

Within this difficult funding environment, could UNDP have made a better case for early recovery? UNDP did try, but was not successful. Readers will recall that early recovery was a component of the essential services pillar of the UNMEER strategy, and was placed under UNMEER's coordinating responsibility by instruction of the Secretary-General. The importance of focusing all resources on Stop and Treat was reiterated by the Secretary-General and by UNMEER on a number of occasions in the September-November timeframe, creating a further **disincentive to UNDP openly advancing an early recovery agenda in the midst of the medical response**. At the same time there was very strong reinforcing NGO pressure on the medical approach. The combined result of all this was that when early recovery advisers arrived in the field in November (late), they found that the doctrine of Stop and Treat was well entrenched. If there had ever been a window to put early recovery on the table in October then that window was already closed, and by November neither host governments nor donors had the interest or energy to think about early recovery.³¹

This was still the situation at the end of November, when the UNDP Administrator hosted an early recovery side event on the margins of the Executive Board meeting. At this well-attended event the first comprehensive statement of goals and objectives was tabled - the **Ebola Crisis Response and Resilience Plan** - **but the donor response to its early recovery elements was weak**; it was simply too early for the donors. In addition, it is inherent in the logic of early recovery that it needs to be led by the host government, and within that frame any UNDP early recovery plan should be in support of a government recovery plan. **Since there was no government early recovery plan in November 2014, UNDP's ideas did not have a context**. By the time the governments did have early recovery plans, it was already too late and the donors had exhausted their reservoirs of emergency funding and were anxious to declare victory and close this chapter entirely. Early recovery was thus caught in a Catch-22 situation: if you push the early recovery button early then it will not be answered, but by the time people are ready to answer it then it is already too late to push the button.

54. THE INERTIA OF THE EMERGENCY MACHINE

There is one further reason why it was difficult to shift attention to early recovery: **the inertia of the emergency machine itself**. In this emergency response there was an 'Ebola economy' in which certain actors (including in host governments) held a measure of 'Ebola power', and their self-interest (consciously or unconsciously) was to maintain the new status quo. Donors did not always help either. At several moments in early 2015, when UNDP was trying to change the channel from response to recovery, and emblematically promoting the message of 'Getting to zero and staying at zero' to provide a hook for recovery thinking, the UK countered with 'Stop and Treat' inertia, to the point that the UK supported Sierra Leone and Liberia to develop alternate national recovery plans to those on which UNDP was working together with its Ministry of Finance counterparts.

²⁹ Disaster preparedness falls into a similar limbo between humanitarian and development mechanisms.

³⁰ The only donor that seems to have provided substantial new development funding is the UK to Sierra Leone – where DfID took a major role in shaping the Government's plan.

³¹ The irony is that the medical response also took time to get going and there was overestimation of the trajectory of needs, such that the medical response was still scaling up massively when the epidemic was already in decline. By mid-2015 there was a situation of substantial medical oversupply, overbuilding and overspending, leaving all other sectors starved for support.

55. THE NEED FOR A NARRATIVE

The most common shortcoming mentioned by respondents in interviews was the **absence of an early clear statement of UNDP's objective and strategy** – a statement that would identify UNDP's comparative advantage and provide a rallying point for planning and messaging.³² Finding this unifying narrative was extraordinarily difficult and UNDP was not alone in struggling with this. As mentioned earlier, a narrative did eventually emerge for the first UNDP donor outreach meeting at the end of November. This was followed by several revisions, necessary as the situation evolved and the focus shifted incrementally from response to recovery. At one point UNDP found itself in the uncomfortable position of having several partial narratives reflecting different moments in time or different levels of response, and felt the need to develop a map of the maps. This was the Achilles heel for programming, which needs a strong framing idea in order to be successful.

The absence of an early clear narrative also placed BERA at a major disadvantage. Because the communications terrain was full of issue-specific and loud voices, starting with UNMEER itself but including heavyweights like UNICEF, the Red Cross, MSF and the US Government/CDC, there was little space for early recovery messaging -- and even today UNDP does not figure significantly in most retrospective reports on the Ebola response.

56. PROJECTS THAT DID NOT CONNECT WITH DONORS

Given the lukewarm donor interest in early recovery and the succession of response-to-recovery strategies, the project teams in HQ and in the field also had an uphill struggle. Those programming initiatives that were successful in the early stages (PPERW and also a series of smaller country-level projects funded by the MPTF) were precisely the ones that did fit within the prevailing mindset of medical response, and had clear champions. **For the most part the recovery proposals were not funded.**³³ Some interlocutors felt that too much energy was spent seeking project funding, at the expense of policy and planning work in support of the host governments. But as observed by a UNDP staffer in interview, in the current funding environment of diminishing core resources, *"we cannot respond without a project."* With the benefit of hindsight, some interlocutors conclude that two strategies might have yielded more success: one being a greater focus on accessing the resources of the MPTF (which ended up as the major source of funding to UNDP projects), and the other being a focus on fewer proposals (there were at least 27 the vast majority unfunded) selected for their alignment with UNDP's recognized comparative advantages and with donor interests.

57. DONOR INTELLIGENCE

For reasons briefly outlined earlier, donors are not very responsive or flexible when approached for development funding in a new area or requiring a quick start-up. Development budgets are usually set by parliaments or Congress and cannot easily be changed, development planning usually takes place within fairly rigid multi-year frameworks, and projects are usually planned a year or two before they start implementation. For these reasons, it is not so useful to know what donors are doing now (which is the result of decisions they made long before), but **it is more important to look upstream at what donors are planning now.**

It is at the planning stage that both donors and host governments need to be integrating recovery into their thinking, so that recovery is already embedded in the strategy when it is later turned into projects. Similarly, when doing donor mapping, the key for UNDP should not be to look for which donors are interested in recovery, but rather to understand which donors are interested in each country in agriculture, or in governance, or in fiscal reform etc., so that **UNDP knows how to frame a recovery argument that fits the donor's thematic preferences,** and thereby increase the prospects of some downstream donor funding.

³² This view is not universally held and some stakeholders have expressed a different view.

³³ The After Action Review only focused on the period until 10 July 2015, funding received after the International Ebola Recovery Conference.

Gathering this sort of donor intelligence is much easier if the UNDP office is located in the same city as the donor office. In countries where the UNDP CD has the opportunity to develop personal relationships with each donor, typically through the local donor coordination machinery of conferences and working groups, then UNDP has much better access to understanding what donors are planning upstream and can better position UNDP for downstream funding. CDs who really know the country and the issues are also the best placed to make persuasive arguments for funding when the time comes. **CDs in the Ebola-affected countries were for the most part at a disadvantage in this respect, because few donors had resident development managers on site.**

HOW CAN UNDP BETTER CONTRIBUTE TO A FUTURE LARGE-SCALE HEALTH CRISIS?

58. STICK TO UNDP'S CORE BUSINESS

While considering the strategic position of UNDP within a future large-scale crisis, especially if it is a crisis like Ebola or another emergency outside the normal development and humanitarian frameworks, then the consensus recommendation from staff interviewed is **that it should be framed from the outset as the consequence of a governance (and/or resilience) failure** – requiring a response from UNDP that addresses its governance origins. With a governance rationale, the strategy and the systems of UNDP's response will fit more naturally with UNDP's comparative advantages and systems.

"Be ready for UNDP to play an "administratively light" support role to other agencies, as quickly as may be required. This system-wide support role should be endorsed and supported by all levels of UNDP in advance of an emergency, and should not be seen as a personal risk for any individual staff that are helpful to the other agencies during an emergency response."
Review respondent.

If there is no basis for a governance (or resilience) business case, and no other strong rationale for UNDP programme engagement, then UNDP should consider limiting its role to supporting other UN agencies (interagency coordination and common services) and not engage as a programming actor.

The content of the response programme will of course depend upon the context, but in any scenario it should **aim to build sustainable national capacity, use existing systems**, and the staff deployed to support the response should have skills that are transferable from a crisis to a development context so that the two phases of a future response can be linked from the start.

59. EARLY RECOVERY

In the humanitarian to development transition, **capacity building was seen by many as a systemic weakness, characterized by one interlocutor as a blind spot, and a natural fit for UNDP**. In this respect, early recovery work that is at the strategic and system level (i.e. planning for early recovery, reorganizing government departments around early recovery, adapting national programmes for early recovery) might yield better results³⁴ than specific and typically small-scale economic recovery projects.

60. CONTINUITY OF CORE GOVERNMENT SERVICES

The experience of PPERW suggests that a niche for UNDP would be to **assure the continuity or the resumption of core government functions during and after an emergency**: for example through maintaining salary payments and communications systems in situations of breakdown. This could be branded as 'governance in emergencies.'

³⁴ More sustainable results and with multiplier benefits, although admittedly these system level results might not be as tangible and donor flag-ready – and therein lies part of the funding problem.

61. DO NOT SKIP STEPS

One of the problems with the Ebola response was the difficulty of developing UNDP projects without the context of a government plan. The sequence matters, and although it is easy to say with hindsight (there were no doubt great pressures to act immediately at the time) the recommendation remains that **UNDP should resist the temptation to skip steps**. A good response starts with field-led analysis, then support for a strong country-led plan (while showing engagement with small focused quick impact projects), on the basis of which UNDP can then build a well-founded medium-term recovery programme that fits UNDP's comparative advantages, and finally develop some projects. Without early investment in information, analysis and data to make a persuasive case for a strategic and large programme in nation rebuilding or recovery, then there is a risk that the programme portfolio will resemble a bundle of disparate projects without context. These steps do not need to be followed in a linear way – elements can be worked on in parallel – but ideally all the pieces are conceptually sequenced.

“Assess UNDP’s comparative advantage as early as possible in an emergency (and again as frequently as needed). If UNDP decides it is well positioned to play a certain role, it should dedicate resources and overcome bureaucratic barriers to ensuring it is rapidly able to deliver on its role. UNDP should establish direct, specific relationships and partnerships more quickly with other key players (e.g. WHO, World Bank) at the country, regional and the global level, which would reduce unnecessary [inter-agency] competition.”
Review respondent.

62. HAVE DIFFERENT TEAMS WORKING ON THE COUNTRY PLANNING AND ON UNDP’S PLANNING

Given that an emergency does not usually offer the luxury of time that this sequenced strategy suggests, UNDP can also work on the national planning track and the UNDP planning track in parallel, perhaps with **different teams and linkages between them**, so that a UNDP plan/programme is ready at the same time as the national plan/programme which it is supporting. This idea of separate teams was also offered as a way of ensuring that a donor conference meets the needs of the affected country (a track that could be led by the RC) as well as UNDP's own resource mobilisation needs (a track that could be led by the CD).

63. HAVE SEED FUNDING AVAILABLE

The recommendation was made earlier in the chapter on deployments, but bears reiterating here, that **having available seed funding allows UNDP to be present and relevant quicker**,³⁵ and in an emergency having something small that is relevant and on time is a lot more effective than something larger but later.

64. RESOURCE MOBILISATION

Resource mobilisation will be facilitated by bringing four components together in a package. The suggested four essential

“Where there is a risk of insufficient funding, strategies would benefit from being phased or sequenced. The ideal scenario would be to have an implementation plan that considers continued fund raising, partnership development and communications built in. Difficult issues (e.g. corruption) should be addressed from the beginning of the recovery programmes in a genuine partnership approach.”
Review respondent.

³⁵ It seems that the new Funding Window for the Emergency Development Response to Crisis and Recovery meets this need.

components³⁶ are: (a) a country-led and universally accepted vision/plan, (b) a specific UNDP idea in an area of UNDP's comparative advantage and that is aligned with the whole, (c) an analysis of the priorities and requirements of likely donors, and (d) if there is not one already, then a funding vehicle such as a trust fund or a joint programme.

Before investing heavily in developing a programme and a suite of related projects, it would be preferable if at the highest levels of UNDP there could be a **closed-door conversation with each of the major donors to determine the prospects of a UNDP programme being funded**. Donors are unlikely to commit support at such a closed door bilateral meeting, especially if it is early and the situation is fast-changing, but they are likely to be able to signal discouragement if necessary, and if they signal encouragement then they can suggest which approaches are more likely to meet their donor priorities.

Finally, if there is an existing vehicle like the MPTF, then UNDP should support its proposals by **engaging with the people who control the funding decisions**, in this case it would have been the Steering Committee of the MPTF, the UN Special Envoy on Ebola, and the key MPTF donors.

³⁶ The last three of these were put in place by BERA/RBA led Resource Mobilisation Task Force mandated by the Crisis Board.

ANNEX 1 – SUMMARY OF RECOMMENDATIONS³⁷

RECOMMENDATIONS TO UNDP SENIOR MANAGEMENT

- **Appoint a senior coordinator at the outset.** Depending on the context, this could simply be the designation of an existing ASG or Director as the “Senior Coordinator”, with accompanying special authorities and responsibilities.
- **Give this senior coordinator the authority and resources** required to implement the mandate, which ideally includes a small staff team, and access to a quick-disbursing source of flexible funding. The coordinator should be positioned close to senior management and close to the operations, which suggests attachment to the Regional Bureau responsible for the affected country. If the crisis is global, then the coordinator could be attached to the Executive Office or to CRU.
- **Ensure field leadership is ready.** Assess the leadership configuration in the field, to assign a senior executive to support the RC if necessary, and to provide immediate SURGE support in areas that are inevitably going to be needed in a major crisis.
- **Determine UNDP’s role,** taking into account other actors and UNDP’s comparative advantages, and develop a clear statement of goals and objectives around which the entire organisation can rally.
- **Identify the key focal points.** The senior coordinator should be well supported and each bureau should identify a key focal point for the emergency – a single gateway for communications, information and tasking so that staff do not receive multiple requests for similar information through different channels.
- **Crisis Board should meet as needed to make Agency-wide decisions,** but should not meet only for information sharing.
- **Get the UNCT organised:** conduct an early orientation of UNCT members on the coordination mechanism, roles and expectations, so all UNCT members are on the same page when it comes to messaging. It might be preferable to expand the UNCT to a UNCT plus, and to co-locate in one facility.
- **Send a high level mission to the field:** this will facilitate (a) a clear strategy with common messaging, (b) risk assessment, (c) assessment of field capacity and priority staffing gaps, and (d) consolidating an initial SURGE response plan.
- **Enhance CRU with some information management and coordination capacity**
- **Establish a quick-response programme funding mechanism** that would allow the field to immediately finance small strategic initiatives that fill critical gaps and secure UNDP’s responsive credibility.

RECOMMENDATIONS ON THE COORDINATION AND MANAGEMENT OF DEPLOYMENTS

- **Keep working on rosters, on updating the SOPs,** and on improving the tools.
- **Keep developing ‘Crisis Response Packages’** that facilitate quick and consistent implementation of core functions while remaining sensitive to context.
- If the Fast Track Procedures still aren’t considered ‘fast enough’ by COs, then **develop a Fast Track 2.0, in particular with regards to Human Resources and Procurement.**
- When crisis strikes, **designate a dedicated human resources professional to support the deployments, as well as boosting the capacity of United Nations Medical Services** to expedite medical clearances.
- The **priority for immediate deployment should be staff rather than consultants,** and should include IM, procurement, finance and human resources support to the affected Country Office.
- **Deploying a few core staff should be done immediately,** as the first people on the ground will have important advantages in terms of knowledge, relationships, access and strategic positioning, and will greatly increase the prospects of UNDP being able to occupy and perform in the coordination space.

³⁷ Note that some of these recommendations have already been partly or fully implemented, in some cases through the revision to the CRU SOPs.

- **SURGE deployees should be seen and managed as a support to the Country Office**, preferably reporting to a Deputy Country Director and integrated into programming and operations systems.
- CRU should **ensure that every deployee has a clear ‘owner’ of the assignment** – a go-to person to troubleshoot the deployee’s problems, to confirm the mandate, to validate work accomplished and to provide ongoing support.
- Ensure an **explicit exit planning process involving the HQ and field stakeholders** several weeks or even months before the expected end of the emergency phase. Require all deployees to write a debriefing report.

RECOMMENDATIONS ON RESOURCE MOBILISATION

- Bring **four components together in a package**: (a) a country-led and universally accepted vision/plan, (b) a specific UNDP idea in an area of UNDP’s comparative advantage and that is aligned with the whole, (c) an analysis of the priorities of likely donors, and (d) a funding vehicle such as a joint programme.
- Before investing heavily in developing a programme **have a closed-door conversation with each of the major donors** to determine the prospects of a UNDP programme being funded, and if they signal encouragement then they can suggest which approaches are more likely to meet their donor priorities.
- If there is an existing vehicle like the MPTF, then UNDP should support its proposals to that funding vehicle by **engaging directly with the people who control the funding decisions**.

RECOMMENDATIONS ON PROGRAMMING

- **Stick to UNDP’s core business**. Whenever appropriate, frame a crisis from the outset as the consequence of a governance (and/or resilience) failure – requiring a response from UNDP that addresses its governance origins.
- If there is no basis for a governance (or resilience) business case, and no other strong rationale for UNDP programme engagement, then UNDP should consider limiting its role to supporting other UN agencies.
- In any scenario the programme should aim to **build sustainable national capacity, use existing systems, and the staff deployed to support the response should have skills that are transferable from a crisis to a development context** so that the two phases of a future response can be linked from the start.
- In the humanitarian to development transition, **capacity building was seen by many as a systemic weakness, and a natural fit for UNDP**.
- **Early recovery work that is at the strategic and system level** (i.e. planning for early recovery, reorganizing government departments around early recovery, adapting national programmes for early recovery) **might yield better results than specific and typically small-scale economic recovery projects**.
- A niche for UNDP could be to **assure the continuity or the resumption of core government functions during and after an emergency: for example through maintaining salary payments and communications systems** in situations of breakdown.
- **Do not skip steps**. A good response starts with field-led analysis, then support for a strong country-led plan (while showing engagement with small focused quick impact projects), on the basis of which UNDP can then build a well-founded medium-term recovery programme that fits UNDP’s comparative advantages, and finally develop some projects.
- **Have different teams working on the country planning and on UNDP’s planning**. UNDP can work on the national planning track and the UNDP planning track in parallel, perhaps with different teams and linkages between them, so that a UNDP plan/programme is ready at the same time as the national plan/programme which it is supporting.
- **Have seed funding available**: having available seed funding allows UNDP to be present and relevant quicker, and in an emergency having something small that is relevant and on time is a lot more effective than something larger but later.

ANNEX 2 – LIST OF PEOPLE CONSULTED

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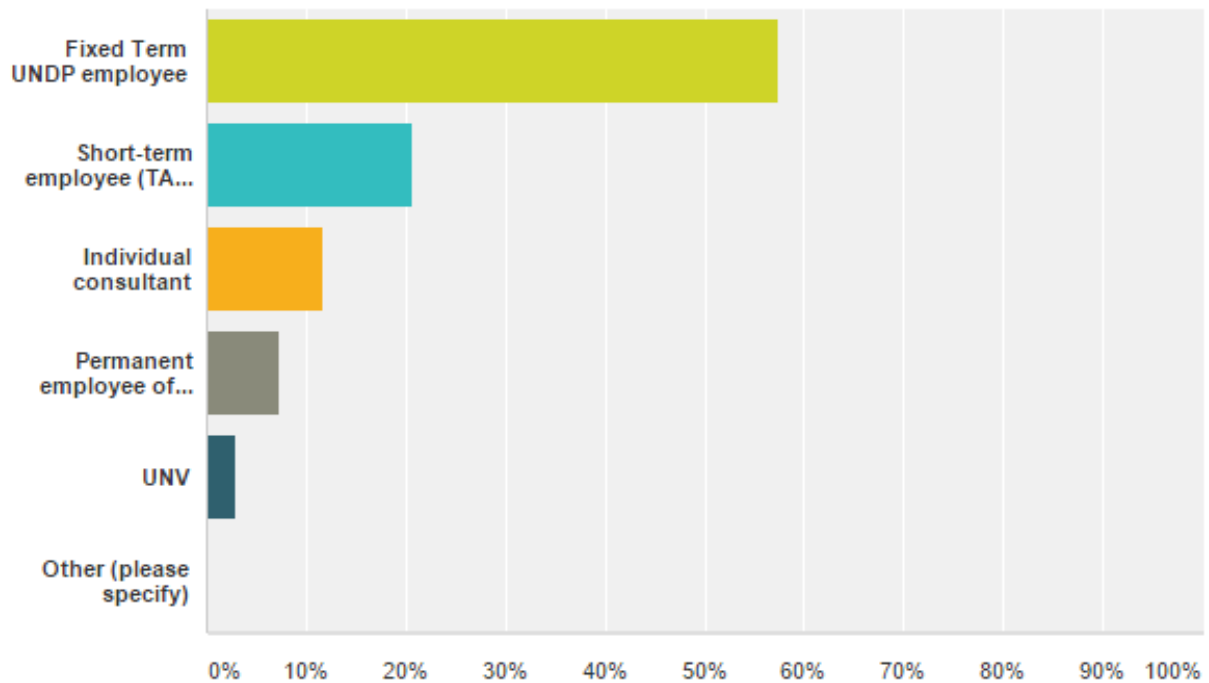
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ANNEX 3 – ONLINE SURVEY RESULTS

What was your contractual status during the Ebola response? (If the status changed, choose the longest period)

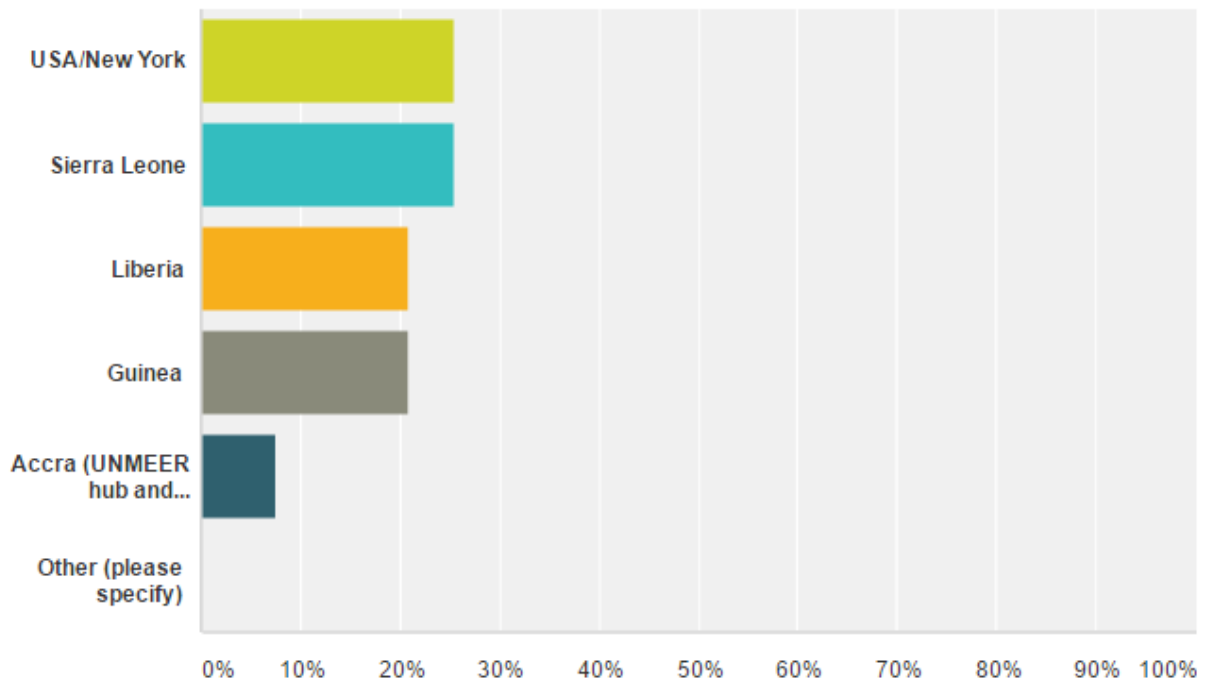
Answered: 68 Skipped: 0



Answer Choices	Responses	
Fixed Term UNDP employee	57.35%	39
Short-term employee (TA contract)	20.59%	14
Individual consultant	11.76%	8
Permanent employee of another UN agency	7.35%	5
UNV	2.94%	2
Other (please specify)	0.00%	0
Total		68

In which location did you do most of your Ebola-related work (this might not be your Duty Station)?

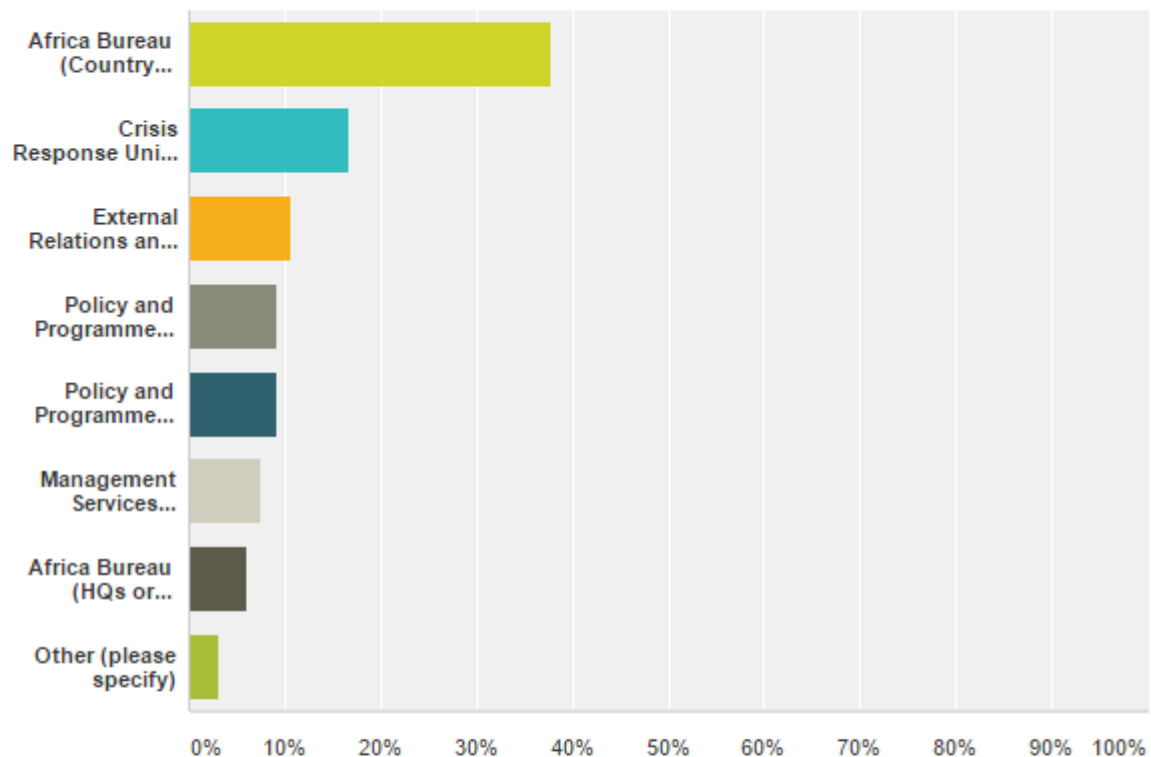
Answered: 67 Skipped: 1



Answer Choices	Responses	
USA/New York	25.37%	17
Sierra Leone	25.37%	17
Liberia	20.90%	14
Guinea	20.90%	14
Accra (UNMEER hub and multi-country support)	7.46%	5
Other (please specify)	0.00%	0
Total		67

Which Bureau/Unit were you working in or attached to during the height of the response?

Answered: 66 Skipped: 2

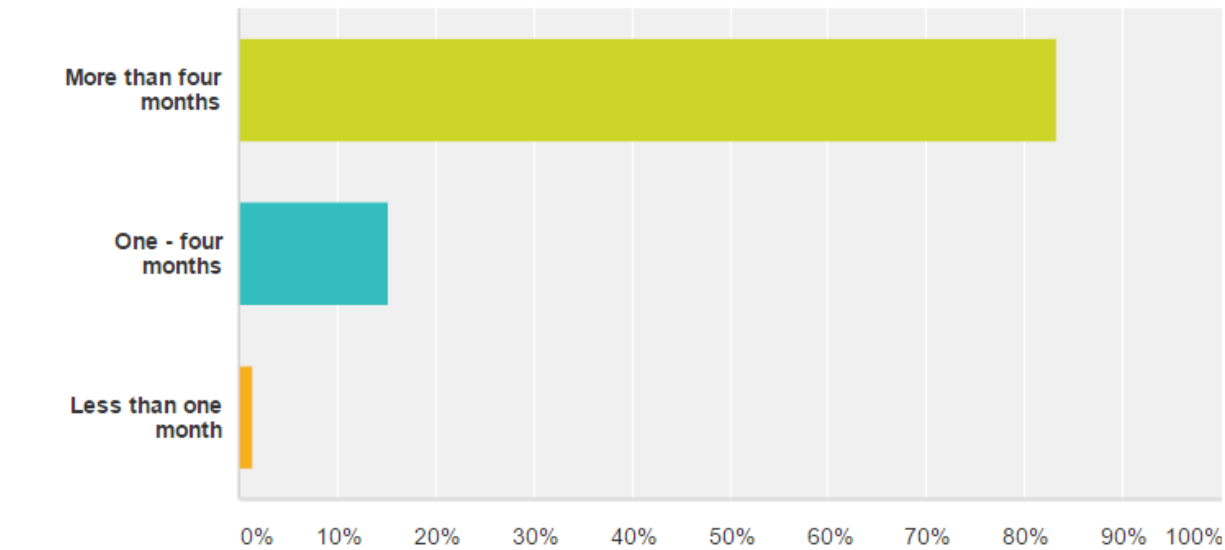


Answer Choices	Responses	
▼ Africa Bureau (Country Office)	37.88%	25
▼ Crisis Response Unit	16.67%	11
▼ External Relations and Advocacy	10.61%	7
▼ Policy and Programme Support	9.09%	6
▼ Policy and Programme Support	9.09%	6
▼ Management Services (formerly BOM)	7.58%	5
▼ Africa Bureau (HQs or Regional Office)	6.06%	4
▼ Other (please specify)	3.03%	2

Responses

How long were you actively involved in the Ebola response?

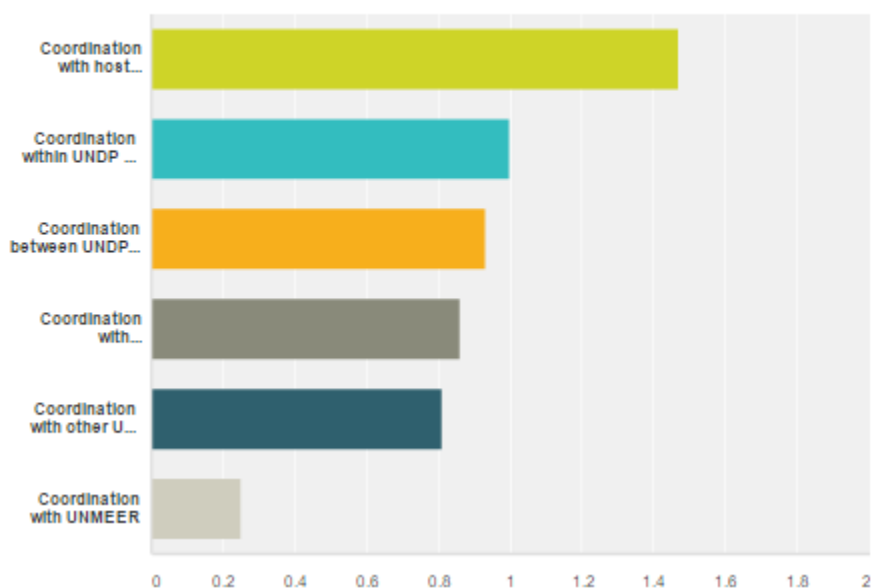
Answered: 66 Skipped: 2



Answer Choices	Responses	
More than four months	83.33%	55
One - four months	15.15%	10
Less than one month	1.52%	1
Total		66

How effective was UNDP's coordination?

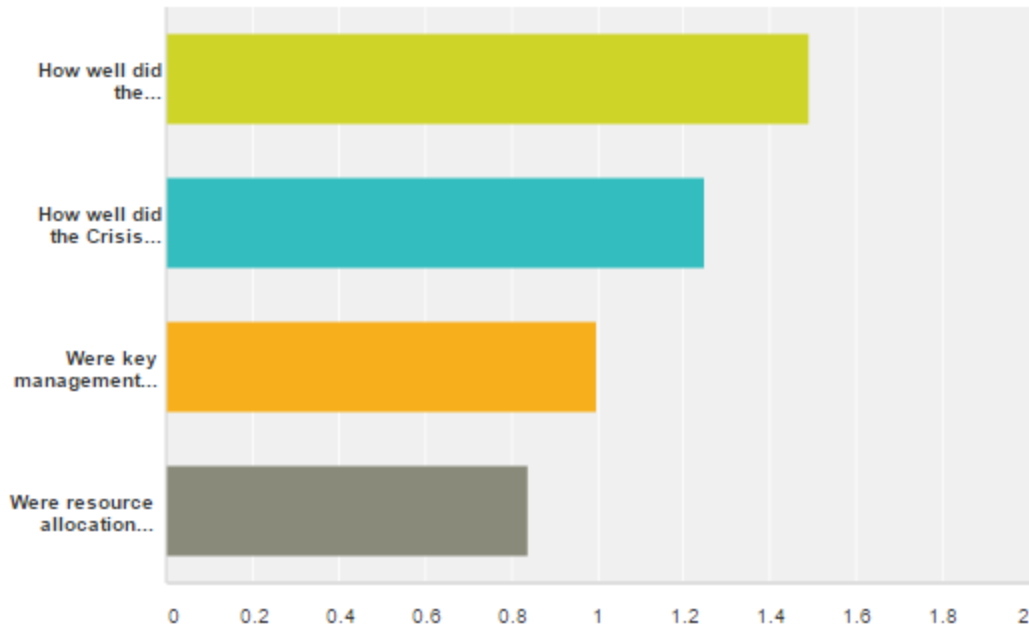
Answered: 66 Skipped: 2



	Poor	Fair	Good	Excellent	Do not know	Total	Weighted Average
Coordination with host Governments	3.03% 2	7.58% 5	45.45% 30	34.85% 23	9.09% 6	66	1.47
Coordination within UNDP HQs (whole of UNDP approach)	4.55% 3	16.67% 11	45.45% 30	25.76% 17	7.58% 5	66	1.00
Coordination between UNDP HQs and UNDP Regional/Country Offices	7.58% 5	18.18% 12	33.33% 22	30.30% 20	10.61% 7	66	0.93
Coordination with donor/support Governments	3.03% 2	16.67% 11	50.00% 33	16.67% 11	13.64% 9	66	0.86
Coordination with other UN agencies and multilateral banks	6.06% 4	18.18% 12	50.00% 33	21.21% 14	4.55% 3	66	0.81
Coordination with UNMEER	9.09% 6	31.82% 21	31.82% 21	16.67% 11	10.61% 7	66	0.25

How timely and efficient was UNDP's management decision-making?

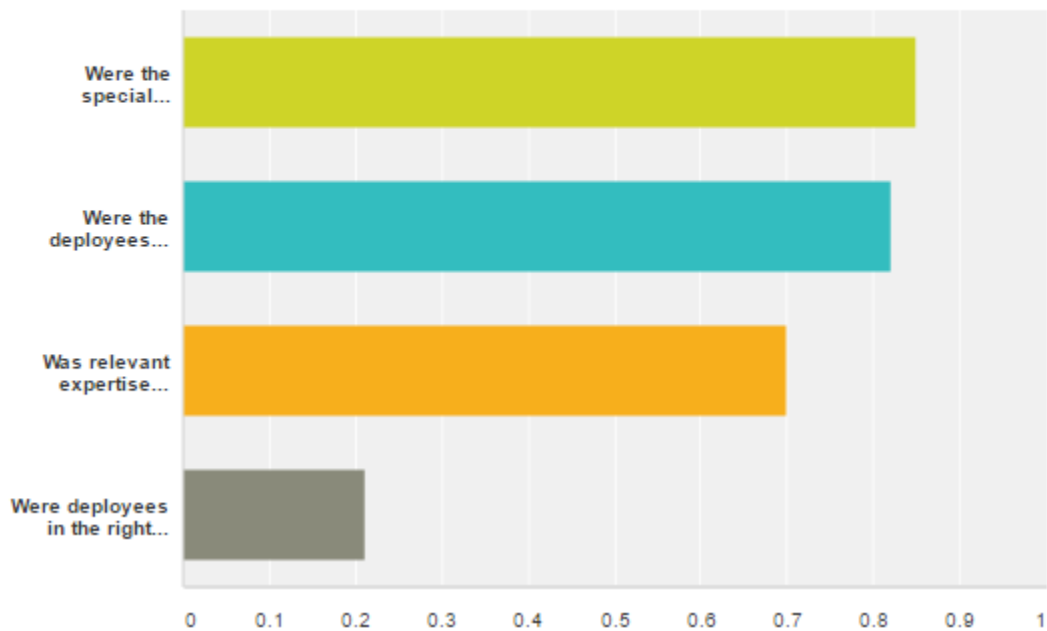
Answered: 66 Skipped: 2



	Poor	Fair	Good	Excellent	Do not know	Total	Weighted Average
How well did the Country-level decision-making function?	0.00% 0	12.12% 8	40.91% 27	33.33% 22	13.64% 9	66	1.49
How well did the Crisis Board function?	1.52% 1	13.64% 9	42.42% 28	27.27% 18	15.15% 10	66	1.25
Were key management decisions timely and clear?	1.52% 1	24.24% 16	40.91% 27	27.27% 18	6.06% 4	66	1.00
Were resource allocation decisions timely and appropriate?	1.52% 1	25.76% 17	45.45% 30	21.21% 14	6.06% 4	66	0.84

How efficient was the staff and expert deployment mechanism?

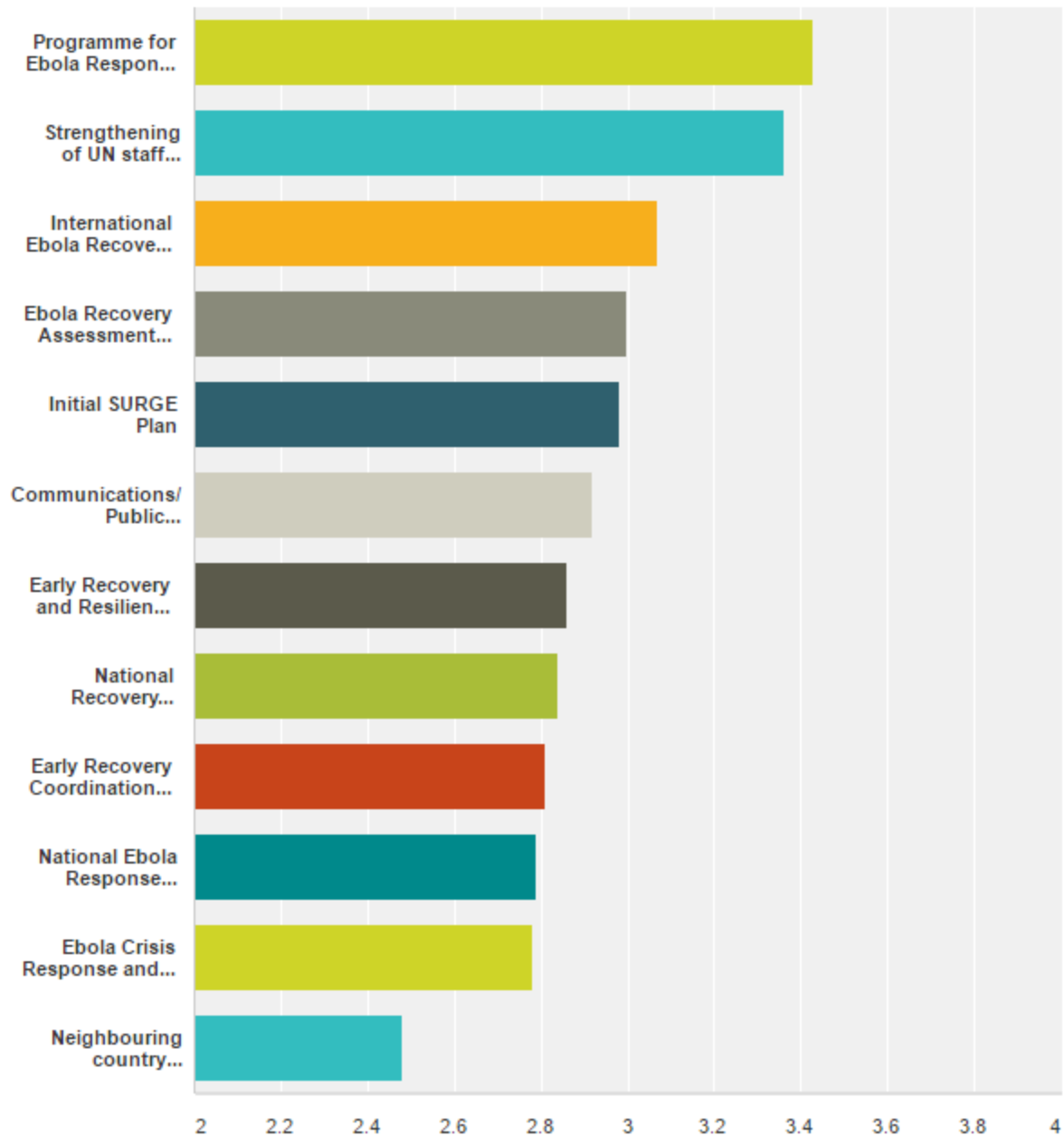
Answered: 66 Skipped: 2



	Rarely	Sometimes	Usually	Always	Total	Weighted Average
Were the special measures for staff and consultant safety appropriate?	4.55% 3	24.24% 16	45.45% 30	25.76% 17	66	0.85
Were the deployees relevant to the needs in the field?	3.03% 2	22.73% 15	54.55% 36	19.70% 13	66	0.82
Was relevant expertise available?	1.52% 1	28.79% 19	53.03% 35	16.67% 11	66	0.70
Were deployees in the right places at the right times?	6.06% 4	33.33% 22	54.55% 36	6.06% 4	66	0.21

In results terms, how effective were each of these UNDP contributions to the Ebola response?

Answered: 66 Skipped: 2



	Poor	Fair	Good	Excellent	Do not know	Total	Weighted Average
Programme for Ebola Response Worker Payments	3.03% 2	7.58% 5	28.79% 19	53.03% 35	7.58% 5	66	3.43
Strengthening of UN staff clinics	4.55% 3	6.06% 4	25.76% 17	43.94% 29	19.70% 13	66	3.36
International Ebola Recovery Conference	4.55% 3	9.09% 6	30.30% 20	22.73% 15	33.33% 22	66	3.07
Ebola Recovery Assessment (Recovering from the Ebola Crisis)	7.58% 5	13.64% 9	36.36% 24	28.79% 19	13.64% 9	66	3.00
Initial SURGE Plan	3.03% 2	12.12% 8	48.48% 32	16.67% 11	19.70% 13	66	2.98
Communications/Public Information documents	9.09% 6	10.61% 7	48.48% 32	21.21% 14	10.61% 7	66	2.92
Early Recovery and Resilience Support Framework (Getting Beyond Zero)	3.03% 2	19.70% 13	36.36% 24	15.15% 10	25.76% 17	66	2.86
National Recovery Strategies	6.06% 4	16.67% 11	45.45% 30	15.15% 10	16.67% 11	66	2.84
Early Recovery Coordination and programming	7.58% 5	16.67% 11	48.48% 32	15.15% 10	12.12% 8	66	2.81
National Ebola Response Strategies	7.58% 5	15.15% 10	50.00% 33	12.12% 8	15.15% 10	66	2.79
Ebola Crisis Response and Resilience Programme	6.06% 4	19.70% 13	43.94% 29	13.64% 9	16.67% 11	66	2.78
Neighbouring country preparedness/contingency	10.61% 7	22.73% 15	33.33% 22	6.06% 4	27.27% 18	66	2.48

ANNEX 4 – FINAL TERMS OF REFERENCE

REVIEW QUESTIONS

The overall purpose of this After Action Review is to examine the efficiency and effectiveness of UNDP's response to the Ebola crisis from September 2014 to July 2015, draw lessons learned, and provide recommendations to UNDP for similar large scale and/or L3 health crises.

Specifically, in response to the Ebola crisis:

- How effective was UNDP's coordination?
- How timely and efficient was UNDP's management decision-making?
- How efficient was the SURGE mechanism?
- How relevant, efficient and effective were the main outputs of the SURGE mechanism?
- How effective was UNDP's communications?

Primary question	Sub-questions (aspects to consider in FGDs and interviews)
How effective was UNDP's coordination?	Coordination with UNMEER Coordination with other UN agencies and IFIs Coordination with host governments Coordination with donor/support governments Coordination within UNDP HQs (whole of UNDP approach) Coordination between UNDP HQs and UNDP Regional/Country Offices
How timely and efficient was UNDP's management decision-making?	How well did the Crisis Board function? How well did the country-level decision-making function? Were key management decisions timely and clear? Were resource allocation decisions timely and appropriate?
How efficient was the staff and expert deployment mechanism?	Was relevant expertise available? Were the deployees relevant to the needs in the field? Were SURGE deployees in the right places at the right times? Which contractual modalities seemed most efficient? Were the special measures for staff and consultant safety appropriate?
How relevant, efficient (return on investment) and effective were the main contributions of UNDP to the Ebola response?	Initial SURGE Plan Ebola Crisis Response and Resilience Programme (ECRRP) Payment Programme for Ebola Response Worker Payments (PPERW) Strengthening of UN staff clinics Ebola Recovery Assessment (Recovering from the Ebola Crisis) Early Recovery and Resilience Support Framework (Getting Beyond Zero) National Response Strategies National Recovery Strategies Neighbouring country preparedness/contingency Early Recovery Coordination and Programming International Ebola Recovery Conference Communications/Public Information documents
How effective were UNDP's external communications and resource mobilisation?	How well did UNDP articulate its role within the overall UN response? How effectively did UNDP advocate for the interests of UNDP? How effectively did UNDP advocate for interests of affected countries? What is the donor assessment of UNDP's Ebola response?

METHODOLOGY

The Ebola AAR will use mixed methods including document review, key informant interviews, approximately six focus group discussions (four in New York and one in each country where field visits are planned³⁸), an online survey, and a validation workshop. The six Focus Group Discussions might use a smartphone-based audience response system to permit a real-time survey to support the discussion and derive some quantitative parameters. UNDP is requested to assist with the following:

1. Provide all relevant documents in a dropbox account. These should include sample CRU AARs, known AARs of other agencies involved in the Ebola response, any back-to-office or end-of-mission reports that might be available, minutes of Crisis Board meetings, internal reports on key events, and other documents recording key business decisions regarding the Ebola response
2. Provide names and contact details of AAR focal points in Freetown, Conakry and Monrovia
3. Provide e-mail addresses of all SURGE deployees
4. Provide names and e-mail addresses of all key HQs staff to be included in focus groups or individual key informant interviews
5. Provide a laptop connected to the internet and a projector/screen in a workshop room for three sessions of 2 hours on 11-12 August
6. Provide webinar facilities (Go to Webinar, Go to Meeting, Adobe Connect, or Webex) that can connect to the three field locations and Geneva, on the morning of 12 September (TBC)
7. Provide a conference room with laptop and projector/screen on the afternoon of 12 September

³⁸ Field visits are planned in Guinea and Sierra Leone; the consultant will collect info and feedback from Liberia remotely and a Focus Group Discussion might be organized.